Patient Information Form

Hospital Name: VALLEY HOSPITAL MEDICAL CENTER **Phone:** (702) 388-4000

Hospital Address: 620 Shadow Ln LAS VEGAS, NV 89106

Patient Demographics

Patient Name: COLE, CAROL G.
Medical Rec #: 63890719 **SSN:**
Marital Status: D **Gender:** F
Date of Birth: 12-28-1944 **Age:** 81
Religion: OTH
Episode ID: 1101309621
Height:
Weight:

Admit and Length of Stay Information

Unit: 3N
Admit Type: EMERGENCY **Room:** 316
Admission Date: 03-25-2026 **Bed:** 01
Est. Discharge Date: 03-25-2026 **ALC Date:**
Pt. Functional Status Prior to Admission:
PCP:
Attending Physician: Techathaveewat, Worawith
Phone Number:

Diagnosis Information:

Rugs: **Primary:** Hypokalemia **Secondary:** Acute kidney failure, unspecified

Discharge Notes:

Notes:

Mode of Transportation:
Will patient receive radiation or dialysis off-site?
☐ Yes ☐ No

Payer Source:
Schedule of Treatments:

Patient Address

Living Arrangement: Select One
Facility Name:
Street: 4720 FAIRFAX AVE
City: LAS VEGAS
State/Zip: NV 89120
Home Phone: (702)451-0591
Work Phone:

Next of Kin

First Name/MI: MINDY
Last Name: ROMAN
Street: 4720 FAIRFAX AVE
City: LAS VEGAS
State/Zip: NV 89120
Home Phone: (702)218-5292
Work Phone:
Relation: K
☐ Emerg. Contact ☐ POA

Emergency Contact:

First Name/MI:
Last Name:
Street:
City:
State/Zip:
Home Phone:
Work Phone:
Relation:
☐ Emerg. Contact ☐ POA

Payer Information:

Ins. Group ID#:
Primary Payer: MEDICARE PART A **Member ID#:** 7PP0DC8GU25
Contact person at Ins. Co. (First/MI/Last): **Phone:**
Patient has met 3 consecutive, acute level of care days during this admission & may be eligible for the Medicare Extended Care Benefit. ☐ Yes ☐ No ☐ N/A ☐ Unknown

Secondary Payer: MEDICAID 2ND MCARE **Member ID#:** 51873700001 **Phone #:**
Other Payer: **Member ID#:** **Phone #:**

Income if known: ☐ Private Funds ☐ SSA ☐ SSI ☐ Pension ☐ VA ☐ Other

Patient Medicaid Eligible? ☐ Yes ☐ No **If Yes, submitted by our financial office?** ☐ Yes ☐ No
Contact person in financial office (First/MI/Last):

No Fault: Must have claim number, name of insurance company, name of insured, telephone number, and policy number. This information is needed even if no fault is exhausted. No faults must have secondary insurance information.

Case Contact

First/MI/Last: Danielle Paine **Phone** (702) 630-6831 **Date:** 03-25-2026

VHM- Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106-4194
(702) 388-4000

Patient Information

Patient Name: COLE, CAROL GASTON
Home Address: 4720 FAIRFAX AVE
LAS VEGAS, NV 891201711
Home: (702) 451-0591
Employer Name: Retired
Employer Phone:
Retire Date: 12/01/1996

Sex: Female **Language:** English
DOB: 12/28/1944 **Race:** White
Age: 81 Years **SSN:** XXX-XX-8550
Religion: Other
Marital Status: Divorced
Contact: Phone Calls and Visitor
EMail: No Email Address

Guarantor Information

Guarantor Name: COLE, CAROL GASTON
Patient's Reltn: Self
Billing Address: 4720 FAIRFAX AVE
LAS VEGAS, NV 891201711
Billing Phone: (702) 451-0591

Sex: Female **DOB:** 12/28/1944
Employer Name: Retired
Employer Phone:

Contact Information

Emergency Contact
Contact Name: MINDY ROMAN
Patient's Reltn: Friend
Sex: Female
Preferred Phone: (702) 218-5292

Next of Kin
Contact Name:
Patient's Reltn:
Sex:
Preferred Phone:

Primary Insurance

Subscriber Name: COLE, CAROL GASTON
Patient's Reltn: Self
Sex: Female
DOB: 12/28/1944

Employer Name: Retired
Employer Phone:
Financial Class: Medicare
Group Name:

Insurance Name: Medicare Part A Only - VHS
Claim Address: MEDICARE PART A
MECHANICSBURG, PA 17055
Insurance Phone: (877) 235-8073
Policy Number: 7PP0DC8GU25
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Secondary Insurance

Subscriber Name: COLE, CAROL GASTON
Patient's Reltn: Self
Sex: Female
DOB: 12/28/1944

Employer Name: Retired
Employer Phone:
Financial Class: Medicaid
Group Name:

Insurance Name: Medicaid Secondary to Medicare -
Claim Address: MEDICAID 2ND MCARE
Reno, NV 89520
Insurance Phone: (877) 638-3472
Policy Number: 51873700001
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Encounter Information

Reg Dt/Tm: 03/23/2026 12:09 PDT
Est Dt of Arrival: 04/01/2026 07:00 PDT
Inpt Adm Dt/Tm: 03/25/2026 10:07 PDT
Disch Dt/Tm:
Observ Dt/Tm:
VIP Indicator:
Admit Reason: HYPOKALEMIA
Accident:
Accident Dt/Tm:

Patient Type: Inpatient
Medical Service: Medicine
Location: VHM 3N
Room/Bed: 316 / 01
Est Responsibility: 0.00

Admit Type: Emergency
Admit Source: Emergency Department
Reg Clerk: Akabutu, Frank
Admitting: Milne, C Dean DO
Attending Techathaveewat DO, Worawith
PCP: Provider Unlisted
Referring: Provider Unlisted
Last Visit Date: 03/23/2026

COLE, CAROL GASTON
Female / 81 Years

MEDICAL RECORD

EL0012

CCPS_REG_XR_FACESHEET Printed By: USER, XR DEFAULT on 03/25/2026 17:01to printer cr_facesheet_12002
Registration last updated by: Cook, Lanisha on 03/25/2026 10:18

Patient Identification
1101309621-63890719
COLE, CAROL GASTON
12/28/1944 81 Ye F
ADM/REG DT: 03/25/2026 10:07

Patient: COLE, CAROL GASTON
MRN: LVMG8010101631; DSH5144223; VHM63890719
DOB/Sex: 12/28/1944 / Female

Admit: 3/23/2026
Disch: Disch Time:
FIN: VHM1101309621
Readmit Risk: 34 - LOW 03/25/26 13:45
Attending: Techathaveewat DO,Worawith

Facesheets

DOCUMENT NAME:	Facesheets
SERVICE DATE/TIME:	3/23/2026 00:00 PDT
RESULT STATUS:	Unauth
PERFORM INFORMATION:	
SIGN INFORMATION:	

FACE SHEET REGISTRATION FORM_20260323.pdf
Please click on link to display result.

Attachment(s): 3/23/2026 00:00 PDT FACE SHEET REGISTRATION FORM_20260323.pdf

VHMC-Cole, Carol Gaston-Enc #1101309621-IPT-MED- FACE SHEET REGISTRATION FORM - 3/23/2026 - 1 pg

MED REC# **63890719**

PT#	PATIENT NAME	CELL PHONE
1101309621	COLE, CAROL GASTON	(702)451-0591

ADDRESS	COUNTY
4720 FAIRFAX AVE	

CITY	STATE	ZIP	PHONE	PATIENT LANGUAGE
LAS VEGAS	NV	891201711	(702)451-0591	ENGLISH

AGE	D.O.B.	SEX	RACE	MS	ETHNICITY	PREVIOUS NAME	RELIGION	INFANT AGE
81Y	12/28/1944	F	1	D	N	COLE, CAROL	OTH	

PT EMPLOYER	TELEPHONE	EXT	OCCUPATION
Retired,			

EMPLOYER ADDRESS

CITY	STATE	ZIP	RETIREMENT DATE	DISASTER TAG ID
			//	

Contact Information

NEAREST RELATIVE NAME	RLTN	ADDRESS

CITY	STATE	ZIP	PHONE	EXT.

EMERGENCY CONTACT NAME	RLTN	ADDRESS
ROMAN, MINDY	K	4720 FAIRFAX AVE

CITY	STATE	ZIP	PHONE	EXT.
LAS VEGAS	NV	891201711	(702)218-5292	

Guarantor Information

GUARANTOR NAME	RLTN
COLE, CAROL GASTON	S

ADDRESS	PHONE
4720 FAIRFAX AVE	(702)451-0591

CITY	STATE	ZIP	OCCUPATION
LAS VEGAS	NV	891201711	

GUARANTOR EMPLOYER	PHONE
Retired	

ADDRESS

CITY	STATE	ZIP

Insurance Information

PC: M

INSURANCE NAME 1	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY
MEDICARE PART A	A00	(877)235-8073	7PP0DC8GU25			

MAILING ADDRESS	CITY	STATE	ZIP
PO BOX 3385	MECHANICSBURG	PA	17055

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM
COLE, CAROL GASTON	MEDICARE PART A,	F	19441228	

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY
1	A00			V	S		

INSURANCE NAME 2	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY
MEDICAID 2ND MCARE	D74	(877)638-3472	51873700001			

MAILING ADDRESS	CITY	STATE	ZIP
PO BOX 30042	Reno	NV	89520

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM
COLE, CAROL GASTON	MEDICAID 2ND MCARE	F	19441228	

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY
2	D74			V	S		

INSURANCE NAME 3	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY

MAILING ADDRESS	CITY	STATE	ZIP

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY

Admitting Information

ADM DATE	TIME	SRC	PT TYPE	PRI	ACCIDENT INFO.	VOLUNTARY ADM	ORGAN DNR	ISOL	PT STS
03/25/2026	1007	EO	E	X	//				X

ATTENDING PHYSICIAN	ATTENDING #	REF SRC	ADM BY	PREV ADM DATE	PREVIOUS FACILITY
Techathaveewat Worawith	939298		VHSCOOKL^		

ADM DIAGNOSIS	HOSP SVC	DIAGNOSTIC CODE	SURGERY DATE	BLDLESS	N/S	RM/BED	Ethnicity
^HYPOKALEMIA	MED		04/01/2026		3N	316 01	N

ADMITTING PHYSICIAN	ADMITTING #	DISCHARGE DATE	TIME	DISCHARGE DISP	PREVIOUS DISCHARGE DATE
Milne C	974121				

PROCEDURE	CLINICAL COMMENT	MODE OF ARRIVAL / ACCOMPANIED BY
		POV^Private Vehicle^VHM_INVISI

REFERRING PHYSICIAN	REFERRING #	EMAIL ADDRESS

REL OF INFO	ADV DIR	LIVING WILL	LOCATION OF WILL / DIRECTIVE	FAMILY DOCTOR	FAMILY DOCTOR #	DIV
						VHM

Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106

EL0012

MEDICAL RECORD

UHS-9002
Rev. 04/2025

Patient Identification

1101309621-63890719

COLE, CAROL GASTON
DOB: 12/28/1944 81Y SX: F MED
MRN: 63890719 ADM/REG DT:03/25/2026
Valley Hospital Medical Center

Page 1 of 1

Patient: COLE, CAROL GASTON Admit: 3/23/2026
MRN: LVMG8010101631; DSH5144223; VHM63890719 Disch:
DOB/Sex: 12/28/1944 / Female FIN: VHM1101309621
Attending: Techathaveewat DO,Worawith

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 3/25/2026 10:11 PDT			
Signed Date/Time: 3/25/2026 10:11 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 3/25/2026 10:11 PDT		End-state Reason:	
Ordering Physician: Orr MD,Trevor		Consulting Physician:	
Entered By: Orr MD,Trevor on 3/25/2026 10:11 PDT			
Order Details: 3/25/26 10:11:00 AM PDT, Routine, Reason for Consult: D/C to Home w Home Health, IV: Yes, O2: No			
Order Comment:			

Orders - Admission/Discharge (Curaspan)

Order: **Update Emergency Contact**			
Order Date/Time: 3/25/2026 15:10 PDT			
Signed Date/Time: 3/25/2026 15:10 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 15:19 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 15:10 PDT			
Order Details: 3/25/26 3:10:47 PM PDT			
Order Comment: Contact Name: ROMAN, MINDY Relationship: daughter Phone: (702) 218-5292			

Action Type: Complete	Action Date/Time: 3/25/2026 15:19 PDT	Action Personnel: Phelps,Shanae
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/25/26 15:10:47 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 15:10 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 15:10:47 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment: Contact Name: ROMAN, MINDY Relationship: daughter Phone: (702) 218-5292		

Orders - Admission/Discharge (Curaspan)

Order: **Status Change**			
Order Date/Time: 3/25/2026 10:07 PDT			
Signed Date/Time: 3/25/2026 10:07 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:17 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:07 PDT			
Order Details: 3/25/26 10:07:38 AM PDT, Admit as Inpatient, Acute Med/Surg with Telemetry			
Order Comment: Status: Admit as Inpatient. Diagnosis: Hypokalemia. Ordered secondary to modification of Status on the Admission/Observation/Transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/25/2026 10:17 PDT	Action Personnel: Cook,Lanisha
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/25/26 10:07:38 PDT, Admit as Inpatient, Acute Med/Surg with Telemetry		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 10:07 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:07:38 PDT, Admit as Inpatient, Acute Med/Surg with Telemetry		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 10:10 PDT Doctor Cosign: Not Required		
Order Comment: Status: Admit as Inpatient. Diagnosis: Hypokalemia. Ordered secondary to modification of Status on the Admission/Observation/Transfer order. 1.00		

Order: **Discharge Request Task Order**			
Order Date/Time: 3/25/2026 10:26 PDT			
Signed Date/Time: 3/25/2026 10:26 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:26 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:26 PDT			
Order Details: 3/25/26 10:26:02 AM PDT, Rehabilitation Unit/Facility			
Order Comment:			

Action Type: Order	Action Date/Time: 3/25/2026 10:26 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:26:02 PDT, Rehabilitation Unit/Facility		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Patient: COLE, CAROL GASTON Admit: 3/23/2026
MRN: LVMG8010101631; DSH5144223; VHM63890719 Disch:
DOB/Sex: 12/28/1944 / Female FIN: VHM1101309621
Attending: Techathaveewat DO,Worawith

Orders - Admission/Discharge (Curaspan)

Order: **Discharge Request Task Order**			
Order Date/Time: 3/25/2026 16:34 PDT			
Signed Date/Time: 3/25/2026 16:34 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 16:34 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 16:34 PDT			
Order Details: 3/25/26 4:34:01 PM PDT, Home with Home Health			
Order Comment:			

Action Type: Order	Action Date/Time: 3/25/2026 16:34 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 16:34:01 PDT, Home with Home Health		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Order: **Discharge Request**			
Order Date/Time: 3/25/2026 10:25 PDT			
Signed Date/Time: 3/25/2026 10:25 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:25 PDT		End-state Reason:	
Ordering Physician: Orr MD,Trevor		Consulting Physician:	
Entered By: Orr MD,Trevor on 3/25/2026 10:25 PDT			
Order Details: 3/25/26 10:25:00 AM PDT, Home with Home Health			
Order Comment:			

Action Type: Modify	Action Date/Time: 3/25/2026 16:33 PDT	Action Personnel: Orr MD,Trevor
Responsible Provider/Electronically Signed by: Orr MD,Trevor	Supervising Provider:	Communication Type: Written
Order Details: 03/25/26 10:25:00 PDT, Home with Home Health		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 16:39 PDT Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 10:26 PDT	Action Personnel: Orr MD,Trevor
Responsible Provider/Electronically Signed by: Orr MD,Trevor	Supervising Provider:	Communication Type: Written
Order Details: 03/25/26 10:25:00 PDT, Rehabilitation Unit/Facility		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 10:29 PDT Doctor Cosign: Not Required		
Order Comment:		

Patient: COLE, CAROL GASTON Admit: 3/23/2026
MRN: LVMG8010101631; DSH5144223; VHM63890719 Disch:
DOB/Sex: 12/28/1944 / Female FIN: VHM1101309621
Attending: Techathaveewat DO,Worawith

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/25/2026 10:07 PDT			
Signed Date/Time: 3/25/2026 10:07 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:18 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:07 PDT			
Order Details: 3/25/26 10:07:38 AM PDT, Attending: Techathaveewat DO, Worawith			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/25/2026 10:18 PDT	Action Personnel: Cook,Lanisha
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/25/26 10:07:38 PDT, Attending: Techathaveewat DO, Worawith		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 10:07 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:07:38 PDT, Attending: Techathaveewat DO, Worawith		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 10:10 PDT Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Patient: COLE, CAROL GASTON | Admit: 3/23/2026
MRN: LVMG8010101631; DSH5144223; VHM63890719 | Disch:
DOB/Sex: 12/28/1944 / Female | FIN: VHM1101309621
Attending: Techathaveewat DO,Worawith

Orders - Admission/Discharge (Curaspan)

Order: **Case Management Discharge Notification**			
Order Date/Time: 3/25/2026 10:26 PDT			
Signed Date/Time: 3/25/2026 10:26 PDT			
Order Status: Canceled	Department Status: Canceled	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 16:34 PDT		End-state Reason: Order Modified	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:26 PDT			
Order Details: 3/25/26 10:26:02 AM PDT			
Order Comment: Rehabilitation Unit/Facility			

Action Type: Cancel	Action Date/Time: 3/25/2026 16:34 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:26:02 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment:		
Action Type: Order	Action Date/Time: 3/25/2026 10:26 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:26:02 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment: Rehabilitation Unit/Facility		

Order: **Case Management Discharge Notification**			
Order Date/Time: 3/25/2026 16:34 PDT			
Signed Date/Time: 3/25/2026 16:34 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 16:34 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 16:34 PDT			
Order Details: 3/25/26 4:34:00 PM PDT			
Order Comment: Home with Home Health			

Action Type: Order	Action Date/Time: 3/25/2026 16:34 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 16:34:00 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment: Home with Home Health		

Physical Therapy Forms

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	3/25/2026 15:10 PDT
RESULT STATUS:	Modified
PERFORMED INFORMATION:	Garcia,Jonathan (3/25/2026 16:13 PDT)
SIGNED INFORMATION:	Garcia,Jonathan (3/25/2026 16:36 PDT); Garcia,Jonathan (3/25/2026 16:13 PDT)

PT Evaluation/Reevaluation Entered On: 3/25/2026 16:28 PDT
Performed On: 3/25/2026 15:10 PDT by Garcia, Jonathan

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 03/25/26 13:50:00 PDT, Routine, Stop date 03/25/26 13:50:00 PDT, Stretcher, Yes, No
Precautions RTF : Active Orders:
» Precautions, 03/23/26 20:06:00 PDT, Routine, Precautions: Seizure, Ordered
» Oxygen Therapy, 03/23/26 18:39:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to maintain SpO2 or SaO2 92-98%, Ordered
» Precautions, 03/23/26 18:39:00 PDT, Routine, Precautions: Fall, Ordered

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate, Calm, Cooperative
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Garcia, Jonathan - 3/25/2026 16:13 PDT

Problem List

(As Of: 3/25/2026 16:28:02 PDT)

Problems(Active)

Acute CHF (SNOMED CT :18472010)	Name of Problem: Acute CHF ; Recorder: Hoppler RN, Sue; Confirmation: Confirmed ; Classification: Medical ; Code: 18472010 ; Contributor System: PowerChart ; Last Updated: 6/9/2015 8:28 PDT ; Life Cycle Date: 6/9/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
Acute pain (SNOMED CT :410499012)	Name of Problem: Acute pain ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 410499012 ; Last Updated: 6/12/2015 4:56 PDT ; Life Cycle Date: 6/12/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/12/2015 4:56 - SYSTEM This problem was added by Discern Expert.

Physical Therapy Forms

Arthritis (SNOMED CT :7278014)	Name of Problem: Arthritis ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 7278014 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 10:43 PDT ; Life Cycle Date: 3/18/2026 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
At risk for falls (SNOMED CT :208683018)	Name of Problem: At risk for falls ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 208683018 ; Last Updated: 6/9/2015 20:42 PDT ; Life Cycle Date: 6/9/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/9/2015 20:42 - SYSTEM This problem was added by Discern Expert.
At risk of pressure sore (SNOMED CT :424268015)	Name of Problem: At risk of pressure sore ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 424268015 ; Last Updated: 6/9/2015 20:36 PDT ; Life Cycle Date: 6/9/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/9/2015 20:36 - SYSTEM This problem was added by Discern Expert.
Bowel dysfunction (SNOMED CT :353134013)	Name of Problem: Bowel dysfunction ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 353134013 ; Last Updated: 6/11/2015 8:24 PDT ; Life Cycle Date: 6/11/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/9/2015 20:36 - SYSTEM Problem added automatically by system based on documentation of abnormal bowel sounds, abnormal GI symptoms, or passing flatus = no.
HTN (hypertension) (SNOMED CT :1215744012)	Name of Problem: HTN (hypertension) ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 1215744012 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 10:43 PDT ; Life Cycle Date: 3/18/2026 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
Ineffective airway clearance (SNOMED CT :112516014)	Name of Problem: Ineffective airway clearance ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 112516014 ; Last Updated: 6/11/2015 19:49 PDT ; Life Cycle Date: 6/11/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments:

Physical Therapy Forms

	6/11/2015 19:49 - SYSTEM Problem added automatically by system based on documentation of inability to clear secretions, abnormal lung sounds, shortness of breath or O2Sat less than 92%.
MRSA (SNOMED CT :176116017)	Name of Problem: MRSA ; Onset Date: 3/24/2026 ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Medical ; Code: 176116017 ; Contributor System: PowerChart ; Last Updated: 3/24/2026 12:47 PDT ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 3/18/2026 14:11 - SYSTEM Problem added by Discern Expert based on Positive MRSA test result 3/19/2026 6:04 - Walsh RN, Timothy F PCR (nares) 3/18/26 3/24/2026 12:47 - Thapar RN, Anusha PCR (nares) 3/24/26
Petit mal seizure status (SNOMED CT :4636990012)	Name of Problem: Petit mal seizure status ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 4636990012 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 10:43 PDT ; Life Cycle Date: 3/18/2026 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
Transfusion history (SNOMED CT :2986959016)	Name of Problem: Transfusion history ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 2986959016 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 11:08 PDT ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 3/18/2026 11:08 - Norwood RN, Yvonne YEAR: 2015.

Diagnoses(Active)

Abnormal laboratory findings	Date: 3/23/2026 ; Diagnosis Type: Reason For Visit ; Confirmation: Complaint of ; Clinical Dx: Abnormal laboratory findings ; Classification: Nursing ; Clinical Service: Non-Specified ; Code: PNED ; Probability: 0 ; Diagnosis Code: 041FFDE2-A217-4FDF-B142-F003962BF651
AKI (acute kidney injury)	Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: AKI (acute kidney injury) ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: N17.9
Hypokalemia	Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: Hypokalemia ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: E87.6

Physical Therapy Forms

Hypokalemia Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: Hypokalemia ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: E87.6

Hypomagnesemia Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: Hypomagnesemia ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: E83.42

Home Environment
Living Environment : Home Environment
Devices/Equipment at Home: Shower chair, Walker, Wheelchair Performed By: Paine RN, Danielle 03/25/2026
Living Situation: Home independently Performed By: Paine RN, Danielle 03/25/2026
Professional Skilled Services: Other: PCA Performed By: Paine RN, Danielle 03/25/2026
Living Situation : Home independently
Lives With : Child(ren)
Lives In : Multilevel home

Garcia, Jonathan - 3/25/2026 16:13 PDT

Location Bed
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Garcia, Jonathan - 3/25/2026 16:13 PDT

Patient's Responsibilities Rehab : Personal ADL

Garcia, Jonathan - 3/25/2026 16:13 PDT

Home Environment II
Living Environment : Home Environment
Devices/Equipment at Home: Shower chair, Walker, Wheelchair Performed By: Paine RN, Danielle 03/25/2026
Living Situation: Home independently Performed By: Paine RN, Danielle 03/25/2026
Professional Skilled Services: Other: PCA Performed By: Paine RN, Danielle 03/25/2026
Home Equipment Rehab : Elevated toilet seat, 4-Wheel walker, Front-wheel walker (FWW), Shower commode chair

Garcia, Jonathan - 3/25/2026 16:13 PDT

Prior Functional Status Grid
ADL : Independent
Mobility : Independent
Instrumental ADL : Independent
Cognitive-Communication Skills : Independent

Garcia, Jonathan - 3/25/2026 16:13 PDT

LE Range/Strength
LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Garcia, Jonathan - 3/25/2026 16:13 PDT

Lt Lower Extremity Strength : Within functional limits
Rt Lower Extremity Strength : Within functional limits

Garcia, Jonathan - 3/25/2026 16:13 PDT

Physical Therapy Forms

Balance
Kansas University Balance Scale : Yes
General Balance : Both
General Balance Assessment : Fair
Sitting Balance Score : 4
Standing Balance Score : 3+

Garcia, Jonathan - 3/25/2026 16:13 PDT

Mobility
Mobility Grid
Roll Left : Rehab Modified independence
Roll Right : Rehab Modified independence
Supine to Sit : Rehab Modified independence
Sit to Supine : Rehab Modified independence
Transfer Sit to Stand : Rehab Modified independence
Transfer Stand to Sit : Rehab Modified independence
Transfer Bed to and From Chair : Rehab Modified independence

Garcia, Jonathan - 3/25/2026 16:13 PDT

Ambulation Level : Close supervision
Device : Gait belt, Front-wheel walker (FWW)
Ambulation Distance : 250 ft
Gait Deviations Left Lower Extremity : Decreased stride length
Gait Deviations Right Lower Extremity : Decreased stride length

Garcia, Jonathan - 3/25/2026 16:13 PDT

Orthostatics
Systolic/Diastolic Sitting BP : 118 mmHg
Systolic/Diastolic Sitting BP : 63 mmHg
Systolic/Diastolic Standing BP : 124 mmHg
Systolic/Diastolic Standing BP : 64 mmHg
Patient Response to Orthostatics : pt denies dizziness/lightheadedness

Garcia, Jonathan - 3/25/2026 16:13 PDT

Assessment
PT Impairments or Limitations : Ambulation deficits, Balance deficits
Barriers to Safe Discharge PT : Limited family support, Medical diagnosis, Severity of deficits, Time since onset

Garcia, Jonathan - 3/25/2026 16:13 PDT

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Garcia, Jonathan - 3/25/2026 16:13 PDT

Physical Therapy Education Grid
Ambulation with Standard Walker : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Bed Mobility : Demonstrates
Bed Positioning : Demonstrates
Bed to Chair Transfers : Demonstrates
Body Mechanics : Demonstrates
Physical Therapy Plan of Care : Demonstrates
Sit to Stand Transfers : Demonstrates

Garcia, Jonathan - 3/25/2026 16:13 PDT

Physical Therapy Forms

Long Term Goals
PT Patient,Caregiver Goal : Pt to get well and go home with family

Garcia, Jonathan - 3/25/2026 16:13 PDT

Short Term Goals
Other PT Goals Grid

	Goal #1
Goal :	Pt to ambulate >150ft Mod I LRAD
Status :	Initial
Timeframe To Complete :	LOS
	Garcia, Jonathan - 3/25/2026 16:13 PDT

Plan
PT Anticipated Treatments, Needs : Balance training, Basic activities of daily living, Bed mobility training, Equipment training, Gait training, Patient education, Posture/Body mechanics training, Stair training, Therapeutic exercises, Transfer training
PT Frequency Rehab : Other: 1+/wk
PT Duration Rehab : 1-2 weeks
PT Intensity Per Day : Other: 1+/wk
Rehabilitation Potential : Good
Post-Acute Discharge Therapy Recommendation PT : Continued intermittent therapy to address functional deficits
IDR PT Recommend/Goal : Pt to d/c home with family and ongoing skilled therapy
PT Plan/Goals Established w Pt/Caregiver : Yes

Garcia, Jonathan - 3/25/2026 16:13 PDT

Additional Information
Additional Information PT : THERAPY SESSION TIME: 53 min
RN CLEARED FOR THERAPY: yes
PAIN LOCATION AND LEVEL: n/a currently
ORIENTATION: AOx4
PLOF: Mod IND

Pt orders received for PT eval, chart reviewed, cleared by RN. Pt presents with hypokalemia; see report for details. Pt was received supine in bed, pleasant and agreeable to PT. Gait belt donned for safety. Pt completed mobility per grid, no LOB, no fall. Pt Mod I for bed mobility, Mod I STS/transfer and SPV ambulation approx 250ft FWW. Pt declined further mobility. PT reviewed walker management and upright posture during mobility, VC for attention to walker placement. Pt would currently benefit from skilled PT during admission to decrease risk for falls and maintain IND. Pt may benefit from skilled PT post d/c to improve upon functional deficits. Pt reports she has assistance at home and equipment. PT rec pt d/c home with ongoing skilled therapy, when medically cleared by physician. Pt was left sitting in chair, per pt request, all needs met, RN updated. Ext time required to complete tasks, rest breaks, slow-steady movements, education. pt declined toileting and hygiene/ADL at this time. chair alarm on

BARRIERS TO SAFE DC/IMPAIRMENTS AND LIMITATIONS: PMH
THERAPY RECOMMENDATION: PT rec pt d/c home with PT, when medically cleared by physician.

Physical Therapy Forms

Environmental Safety Implemented : Bed in low position, Call device within reach, Lock wheeled equipment, Personal items within reach

Garcia, Jonathan - 3/25/2026 16:13 PDT

PT Non Timed Modalities 2026
PT Treatment Start Time : 3/25/2026 15:10 PDT
PT Treatment End Time : 3/25/2026 16:03 PDT

Garcia, Jonathan - 3/25/2026 16:13 PDT

~~PT Eval Moderate Complexity Total Number Min : 1 [IN ERROR]~~

~~{ [1] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

~~PT Total Non Timed : 1 [IN ERROR]~~

Garcia, Jonathan - 3/25/2026 16:36 PDT

~~{ [1] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

PT Timed Modalities 2026
PT Timed Modality Treatment Start Time : 3/25/2026 15:10 PDT
PT Timed Modality Treatment End Time : 3/25/2026 16:03 PDT
Rehab Patient : No

Garcia, Jonathan - 3/25/2026 16:13 PDT

PT Timed Min Total : 53

~~{ [38] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

PT Timed Unit Total : 4

~~{ [3] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

PT THERAPEUTIC ACT 15MIN : 30

~~{ [15] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

Activity Therapeutic Num of Units : 2

Garcia, Jonathan - 3/25/2026 16:36 PDT

~~{ [1] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

HOME/FAMILY INSTRUCT 15MIN : 8
Home/Family Instruct Num of Units : 1
PT GAIT TRAINING 15MIN : 15
Gait Training Num of Units : 1

Garcia, Jonathan - 3/25/2026 16:13 PDT

History and Physical

DOCUMENT NAME:	History & Physical
SERVICE DATE/TIME:	3/24/2026 00:05 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Leng MD,Andrew (3/23/2026 19:40 PDT)
SIGN INFORMATION:	Techathaveewat DO,Worawith (3/24/2026 19:21 PDT); Rahmanzai DO,Sofia (3/24/2026 06:22 PDT); Leng MD, Andrew (3/23/2026 19:54 PDT); Leng MD,Andrew (3/23/2026 19:44 PDT); Leng MD,Andrew (3/23/2026 19:40 PDT)

Chief Complaint/Reason for Consultation
Pt came via walking from pre op c/o low potassium and needing hysterectomy at 0600 this morning. Pt was sent here due to K of 2.4. Pt agitated in triage and only wants food.

Histories
Allergies
Allergies (Active and Proposed Allergies

History and Physical

Pt has no medical complaints. Pt refused EKG.
Pt AxO4 and ambulatory with personal walker.

History of Present Illness
Patient is a 81-year-old female with past medical history of abnormal uterine bleeding and a fib on anticoagulation, was scheduled for hysterectomy was found to have a low potassium at her PCP prior to scheduled surgery was brought to Valley Hospital Medical Center on 3/23/2026 to treat hypokalemia. Prior to her surgery scheduled today was told to follow-up with her PCP. When she followed up with her PCP she was told she had low potassium and needed to go to the hospital to replete it with IV. The GynOnc surgeon that was scheduled for the surgery is Dr. Fleury, who originally wanted to reschedule the surgery for Wednesday, however due to scheduling patient was told that the surgery needed to be postponed possibly by next week. Patient has been admitted to Valley Hospital Medical Center to replete potassium. Patient is asymptomatic denies shortness of breath, chest pain.

Vital signs upon admission were 97.3 °F, 138/63 mmHg, 68 bpm, 100% SpO2 on room air and 16 breaths/min. Significant labs included blood glucose of 122, potassium of 2.4, creatinine 1.31, TSH of 5.146 UA squamous epithelium of 10, 2+ bacteria, 2+ leukocyte esterase. EKG showed no acute pulmonary process. EKG showed atrial fibrillation with ST and T wave abnormalities.

PMHx: CHF, Hypertension, Seizures due to prior PEA in 1994
PSHx: Bilateral Lumpectomy for Breast Cancer (Do not place blood pressure cuff on Left Arm due to lymphadenopathy)
Meds: Carbamazepine 200 mg twice daily, Keppra 750 mg twice daily, clopidogrel 75 mg daily, ipratropium 21 mcg/inh twice daily, metoprolol tartrate 25 mg twice daily, Olmesartan 40 mg daily
Allergies: Adhesive bandages
SocHx: Denies cigarette smoking, denies alcohol use, denies illicit drug use
FamHx: Noncontributory

Review of Systems
Constitutional: No weight loss, fever, chills, weakness or fatigue.
HEENT: No visual loss, blurred vision, double vision or yellow sclera. No hearing loss, sneezing, congestion, runny nose or sore throat.
Skin: No rash or itching.
Cardiovascular: No chest pain, chest pressure or chest discomfort. No palpitations or pedal edema.
Respiratory: No shortness of breath, cough or sputum production.
Gastrointestinal: Occasional Diarrhea No anorexia, nausea or vomiting. No abdominal pain or blood in stool.
Genitourinary: No burning micturition. No urinary frequency or incontinence.
Neurologic: No headache, dizziness, syncope, unilateral weakness, ataxia, numbness or tingling in the extremities. No change in bowel or bladder control.
Musculoskeletal: No muscle pain, back pain, joint pain or stiffness.
Hematologic: No bleeding or bruising.
Lymphatics: No enlarged lymph nodes.
Psychiatric: No depression or anxiety.
Endocrine: No reports of sweating. No cold or heat intolerance. No polyuria or polydipsia.

Objective

Vital Signs (24 hrs)	Last Charted	Minimum	Maximum

Only)
Adhesive Bandage (Severity: Moderate, Onset: Unknown)
Reactions: Itching of skin

Past Medical History
Active Problems(6)
Acute CHF
Arthritis
HTN (hypertension)
MRSA
Petit mal seizure status
Transfusion history

Past Surgical History
RIGHT SHOULDER REVERSE TOTAL ARTHROPLASTY: 2024
RIGHT BREAST LUMPECTOMY: 2019
BOSTON SCIENTIFIC ACID: 01/31/14
RIGHT TOTAL KNEE ARTHROPLASTY: 2004
BACK, BUTTOCK, LEGS AND ARMS BURN GRAPHS: 1994
LEFT PARTIAL MASTECTOMY W/ AXILLARY LYMPH NODE DISSECTION SURGERY: 1994

Social History
Alcohol
Risk Assessment: Denies Alcohol Use; Details: Denies
Home/Environment
Details: Lives with Relative. Living situation: Home/Independent.
Substance Abuse
Risk Assessment: Denies Substance Abuse; Details: Denies
Tobacco
Risk Assessment: Denies Tobacco Use; Details: Never (less than 100 in lifetime)

Family History
No Family History documented.

Medications
Home Medications
carBAMazepine (carBAMazepine 200 mg

History and Physical

Temp	**L** 36.3	03/23/2026 12:19	**L** 36.3	03/23/2026 12:19	**L** 36.3	03/23/2026 12:19
Peripheral Pulse Rate	68	03/23/2026 12:19	68	03/23/2026 12:19	68	03/23/2026 12:19
Resp Rate	16	03/23/2026 12:19	16	03/23/2026 12:19	16	03/23/2026 12:19
SBP	**H** 138	03/23/2026 12:19	**H** 138	03/23/2026 12:19	**H** 138	03/23/2026 12:19
DBP	63	03/23/2026 12:19	63	03/23/2026 12:19	63	03/23/2026 12:19
MAP	91	03/23/2026 12:19	91	03/23/2026 12:19	91	03/23/2026 12:19
SpO2	100	03/23/2026 12:19	100	03/23/2026 12:19	100	03/23/2026 12:19
O2 Therapy	Room air		Room air		Room air	

Intake and Output (current encounter)

No Data Available

Precautions
Fall Precautions

Physical Exam
General: 81yo Female laying in bed in no acute distress
Mental Status: Alert and oriented to person, place and time. Normal affect.
HEENT: Normocephalic, Atraumatic. PERRL. EOMI. Normal dentition. Oropharynx clear, mucous membranes moist. Ears and nose without lesions or deformities. Full range of motion. Trachea midline.
Respiratory: CTA bilaterally, no crackles, rales, wheezes, rhonchi.
Cardiovascular: Regular rate and rhythm, no murmurs, rubs or gallops, normal S1, S2, no peripheral edema.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Bowel sounds normal.
Genitourinary: No CVA tenderness, No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment/Plan
Assessment:
Asymptomatic Hypokalemia
Hypertension
Normocytic anemia 2/2 iron deficiency
Hyperglycemia
Elevated anion gap
AKI
Azotemia
Hypomagnesemia
Subclinical hypothyroidism
Contaminated UA
Previously MRSA positive
Hx of Seizures

oral tablet) 200 Milligram By Mouth 2 Times a Day Indication: SEIZURES
clopidogrel (clopidogrel 75 mg oral tablet) 75 Milligram By Mouth Daily
ipratropium nasal (ipratropium 21 mcg/inh (0.03%) nasal spray) 21 MCG / INH Nasal Inhalation 2 Times a Day
levETIRAcetam (levETIRAcetam 750 mg oral tablet) 750 Milligram By Mouth 2 Times a Day Indication: SEIZURES
metoprolol (metoprolol tartrate 25 mg oral tablet) 25 Milligram By Mouth 2 Times a Day
olmesartan (olmesartan 40 mg oral tablet) 40 Milligram By Mouth Daily

Inpatient Medications
Medications (24) Active
Scheduled: (9)
carBAMazepine 200 mg Tab (carBAMazepine 200 mg oral tablet) 200 mg 1 Tabs, Oral, BID
Heparin Protocols (Heparin Protocol - ACS) Protocol, IV, Daily
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro (HumaLOG) 1 unit/0.01 mL (10 mL) (insulin lispro (HumaLOG) Correctional Scale Low) 1-6 Units, SubCutaneous, Before Meals
levETIRAcetam 750 mg Tab (Keppra) 750 mg 1 Tabs, Oral, BID
losartan 25 mg Tab (losartan) 25 mg 1 Tabs, Oral, Daily
magnesium sulfate/SWFI (magnesium sulfate IV) 2 gm 50 mL, IV Piggyback, Once
metoprolol tartrate 25 mg Tab (Metoprolol Tartrate) 25 mg 1 Tabs, Oral, BID
potassium chloride/SWFI (potassium chloride) 10 mEq 100 mL, IV Piggyback, q1H Interval
Continuous: (2)
Electrolyte (Isolyte-S, Plasma-Lyte) PH7.4 1,000 mL (Isolyte S PH 7.4 1,000 mL) 1,000 mL, IV, 100 mL/hr
heparin/D5W 25,000 units [12 units/kg/hr] + Premix D5W 250 mL (heparin IV additive 25,000 units [12 units/kg/hr] + Premix D5W 250 mL) 250 mL, IV, 6.6 mL/hr
PRN: (13)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q4H

History and Physical

Hx of a fib on anticoagulation
CHF

Plan

Patient is a 81-year-old female with past medical history of abnormal uterine bleeding and a fib on anticoagulation, was scheduled for hysterectomy was found to have a low potassium at her PCP prior to scheduled surgery was brought to Valley Hospital Medical Center on 3/23/2026 to treat hypokalemia. Patient is asymptomatic denies shortness of breath, chest pain.

Admit to med/tele with vital checks every 4 hours

Replete potassium with IV and oral route, will obtain a repeat potassium level at 12:00 later tonight.
Will suspend Clopidogrel today, if she does not have surgery tomorrow morning, will unsuspend and continue her current regimen on plavix.
Preop labs already ordered on 3/18/2026
Insulin sliding scale to treat hyperglycemia, hypoglycemic protocol in place, A1c pending
Beta-hydroxybutyrate, lactate level, VBG pending for mildly elevated anion gap
Fluid resuscitation for AKI suspect prerenal, urine studies pending
Current UA significant for UTI, however seems contaminated, will repeat UA to assess if UTI is real, patient denying dysuria or hematuria, will escalate antibiotics if repeat UA comes back positive
On prior visit, patient was positive for MRSA PCR, will repeat MRSA PCR.

Continue home medications as appropriate
IV PRN antihypertensive medications
Replace electrolytes as needed
Daily labs
Supportive care

Dispo: Med-Tele
Consults: None
Activity: Early and progressive
Diet: Cardiac
Code status: Full
VTE prophylaxis: SCDs and heparin gtt

Andrew Leng, MD
Internal Medicine, PGY-1
Valley Hospital Medical Center
Pager: You call MD using my name or "resident, IM"

Case to be discussed with attending physician, further recommendations to follow.

Patient is an 81-year-old female with past medical history of paroxysmal A-fib on Plavix, CHF status post AICD, hypertension, seizure disorder on antiseizure meds who presented Valley 3/23/2026 for evaluation of hypokalemia. Patient had plan for surgery today with gyn-onc, plan for hysterectomy due to vaginal bleeding. Patient's potassium was initially 2.4, most recent 2.8. EKG was completed and did not show flattened T

calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
naloxone 0.4 mg/mL Inj 1 mL (naloxone) 0.4 mg 1 mL, IV Push, As Directed
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
polyethylene glycol 3350 Oral Pwdr 17 gm pkt (MiraLax) 17 gm, Oral, Daily
simethicone 80 mg Chew Tab (Mylicon) 80 mg 1 Tabs, Oral, q6H

Results
Recent Labs
Antibody Screen

ABSC SP 3 I	Positive (Normal)	03/23/2026 06:57

Blood Group and Type

ABO+Rh I	A POS (Unknown)	03/23/2026 06:57

Cardiac

Troponin TNIH	18.31 ng/L (Normal)	03/23/2026 16:18

General Chemistry

Glucose Level	122 mg/dL (High)	03/23/2026 16:18
POC Glucose	120 mg/dL (High)	03/23/2026 06:52
Sodium	143 mmol/L (Normal)	03/23/2026 16:18
Potassium	2.8 mmol/L (Critical)	03/23/2026 16:18
Chloride	105 mmol/L (Normal)	03/23/2026 16:18
CO2	21 mmol/L (Normal)	03/23/2026 16:18
Anion Gap	17 mmol/L (High)	03/23/2026 16:18

History and Physical

waves or U waves. It appears to be unchanged from EKG presented by her daughter in the morning. Patient denies chest pain, shortness of breath, dizziness, presyncope. She states that she has some abdominal/suprapubic cramping due to the vaginal bleeding. Will replace potassium with IV and oral as tolerated and repeat values. Will keep patient n.p.o. in case of surgery rescheduled for 3/24/2026, however daughter mentioned that the surgeon had to reschedule for next Wednesday, next week. Regarding anticoagulation for paroxysmal A-fib, patient states she was not on an anticoagulant due to severe GI bleeding in the past. She has been off of Xarelto for a while and has been taking Plavix since. She denies history of MI or stents.

Will discuss with attending physician, further recommendations to follow.

Sofia Rahmanzai, DO
Internal Medicine PGY2

Discharge Planning:

Attending addendum: I was physically present and involved in the discussion and management of the patient by the resident, and during critical portions of assessment and services furnished.

Worawith Techathaveewat, DO
Internal Medicine

BUN	19 mg/dL (High)	03/23/2026 16:18
Creatinine	1.31 mg/dL (High)	03/23/2026 16:18
BUN/Creat Ratio	14 (Normal)	03/23/2026 16:18
Calcium	8.7 mg/dL (Normal)	03/23/2026 16:18
Albumin. Level	4.2 gm/dL (Normal)	03/23/2026 16:18
TP	6.6 gm/dL (Normal)	03/23/2026 16:18
A/G Ratio	1.7 (Normal)	03/23/2026 16:18
T Bili	0.3 mg/dL (Normal)	03/23/2026 16:18
Alk Phos	78 units/L (Normal)	03/23/2026 16:18
AST	24 units/L (Normal)	03/23/2026 16:18
ALT	10 units/L (Low)	03/23/2026 16:18
Mg Lvl	1.7 mg/dL (Low)	03/23/2026 14:58
Estimated Creatinine Clearance	29.08 mL/min ()	03/23/2026 16:55
eGFR Cr	41 mL/min/1.73m2 (N/A)	03/23/2026 16:18
eGFR Pediatric	Not Reported (N/A)	03/23/2026 16:18

General Hematology

WBC	9.62 x10e3/mcL (Normal)	03/23/2026 14:58
RBC	3.65 x10e6/mcL (Normal)	03/23/2026 14:58
Hgb	10.6 gm/dL (Low)	03/23/2026 14:58
Hct	34.3 % (Normal)	03/23/2026 14:58
MCV	94.0 Femtoliters (Normal)	03/23/2026 14:58
MCH	29.0 pg (Normal)	03/23/2026 14:58
MCHC	30.9 gm/dL (Normal)	03/23/2026 14:58
RDW-SD	50.0 Femtoliters (High)	03/23/2026 14:58
RDW-CV	14.6 % (Normal)	03/23/2026 14:58
Plt	302 x10e3/mcL (Normal)	03/23/2026 14:58
MPV	9.7 Femtoliters (Normal)	03/23/2026 14:58

History and Physical

Neut % Auto	81.7 % (N/A)	03/23/2026 14:58
Lymph % Auto	10.3 % (N/A)	03/23/2026 14:58
Mono % Auto	6.2 % (N/A)	03/23/2026 14:58
Eos % Auto	0.6 % (N/A)	03/23/2026 14:58
Baso % Auto	0.8 % (N/A)	03/23/2026 14:58
Immature Grans %	0.4 % (Normal)	03/23/2026 14:58
Neut # Auto	7.85 x10e3/mcL (High)	03/23/2026 14:58
Lymph # Auto	0.99 x10e3/mcL (Normal)	03/23/2026 14:58
Mono # Auto	0.60 x10e3/mcL (Normal)	03/23/2026 14:58
Eos # Auto	0.06 x10e3/mcL (Normal)	03/23/2026 14:58
Baso # Auto	0.08 x10e3/mcL (Normal)	03/23/2026 14:58
Immature Grans # Auto	0.04 x10e3/mcL (Normal)	03/23/2026 14:58

Pre-Admission Testing Questionnaire

<3mon: Preg	N/A (Normal)	03/23/2026 06:40
<3mon: Txf/Major Surg	No (Normal)	03/23/2026 06:40

Thyroid

T4 Free	1.14 ng/dL (Normal)	03/23/2026 14:58
TSH Ultrasensitive	5.146 mc Intl units/mL (High)	03/23/2026 14:58

UA Macroscopic

Ur Color	Amber (Abnormal)	03/23/2026 14:58
Ur Clarity	Cloudy (Abnormal)	03/23/2026 14:58
UR Glucose	Negative (Normal)	03/23/2026 14:58
Ur Bili	Negative (Normal)	03/23/2026 14:58
Ur Ketones	Negative (Normal)	03/23/2026 14:58
Ur Spec Grav	1.018 (Normal)	03/23/2026 14:58
Ur Blood	Negative (Normal)	03/23/2026 14:58
Ur pH	5.0 (Normal)	03/23/2026 14:58
Ur Protein	2+ (Abnormal)	03/23/2026 14:58

History and Physical

Ur Urobilinogen	<2.0 mg/dL (Normal)	03/23/2026 14:58
Ur Nitrite	Negative (Normal)	03/23/2026 14:58
Ur Leuk Est	2+ (Abnormal)	03/23/2026 14:58
UA Culture?	No (N/A)	03/23/2026 14:58
Signs/Symptoms of UTI?	No (Normal)	03/23/2026 14:58
UTI Other:	abdominal pain (N/A)	03/23/2026 14:58

UA Microscopic

UA WBC Inst	62 /HPF (High)	03/23/2026 14:58
UA RBC Inst	4 /HPF (High)	03/23/2026 14:58
Ur Bacteria	2+ /HPF (Abnormal)	03/23/2026 14:58
UA Squam Epi Inst	10 /HPF (High)	03/23/2026 14:58
Ur Mucous	Rare /LPF (Abnormal)	03/23/2026 14:58
Ur Hyal Cast	1 /LPF (Normal)	03/23/2026 14:58

Abnormal Labs

General Chemistry

Glucose Level	122 mg/dL (High)	03/23/2026 16:18
POC Glucose	120 mg/dL (High)	03/23/2026 06:52
Potassium	2.8 mmol/L (Critical)	03/23/2026 16:18
Anion Gap	17 mmol/L (High)	03/23/2026 16:18
BUN	19 mg/dL (High)	03/23/2026 16:18
Creatinine	1.31 mg/dL (High)	03/23/2026 16:18
ALT	10 units/L (Low)	03/23/2026 16:18
Mg Lvl	1.7 mg/dL (Low)	03/23/2026 14:58

General Hematology

Hgb	10.6 gm/dL (Low)	03/23/2026 14:58
RDW-SD	50.0 Femtoliters (High)	03/23/2026 14:58
Neut # Auto	7.85 x10e3/mcL (High)	03/23/2026 14:58

History and Physical

Thyroid

TSH Ultrasensitive	5.146 mc Intl units/mL (High)	03/23/2026 14:58

UA Macroscopic

Ur Color	Amber (Abnormal)	03/23/2026 14:58
Ur Clarity	Cloudy (Abnormal)	03/23/2026 14:58
Ur Protein	2+ (Abnormal)	03/23/2026 14:58
Ur Leuk Est	2+ (Abnormal)	03/23/2026 14:58

UA Microscopic

UA WBC Inst	62 /HPF (High)	03/23/2026 14:58
UA RBC Inst	4 /HPF (High)	03/23/2026 14:58
Ur Bacteria	2+ /HPF (Abnormal)	03/23/2026 14:58
UA Squam Epi Inst	10 /HPF (High)	03/23/2026 14:58
Ur Mucous	Rare /LPF (Abnormal)	03/23/2026 14:58

Electronically Signed By: Leng, Andrew MD
On: 03.23.2026 19:40 PDT

Electronically Signed On: 03.23.2026 19:44 PDT
Leng, Andrew MD

Electronically Signed On: 03.23.2026 19:54 PDT
Leng, Andrew MD

Electronically Signed On: 03.24.2026 06:22 PDT
Rahmanzai, Sofia DO

Electronically Signed On: 03.24.2026 19:21 PDT
Techathaveewat, Worawith DO

Discharge Summary

DOCUMENT NAME:	Discharge Summary
SERVICE DATE/TIME:	3/25/2026 10:31 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Orr MD,Trevor (3/25/2026 10:32 PDT)
SIGN INFORMATION:	Techathaveewat DO,Worawith (3/25/2026 11:42 PDT); Orr MD,Trevor (3/25/2026 10:32 PDT)

Patient Information
Attending Physician: Techathaveewat DO, Worawith
Admitting Diagnosis: Hypokalemia
Discharge Location: VHM 3N
Primary Care Physician:
Inpatient Admit Date/Time: 03/25/2026 10:07

Discharge Diagnosis
Hypokalemia (E87.6)
AKI (acute kidney injury) (N17.9)
Hypomagnesemia (E83.42)
Hypokalemia (E87.6)

Discharge Medications
carBAMazepine (carBAMazepine 200 mg oral tablet) 200 Milligram By Mouth 2 Times a Day Indication: SEIZURES
clopidogrel (clopidogrel 75 mg oral tablet) 75 Milligram By Mouth Daily
ipratropium nasal (ipratropium 21 mcg/inh (0.03%) nasal spray) 21 MCG / INH Nasal Inhalation 2 Times a Day
levETIRAcetam (levETIRAcetam 750 mg oral tablet) 750 Milligram By Mouth 2 Times a Day Indication: SEIZURES
metoprolol (metoprolol tartrate 25 mg oral tablet) 25 Milligram By Mouth 2 Times a Day
olmesartan (olmesartan 40 mg oral tablet) 40 Milligram By Mouth Daily

Inpatient Medications
Medications (23) Active
Scheduled: (7)
carBAMazepine 200 mg Tab (carBAMazepine 200 mg oral tablet) 200 mg 1 Tabs, Oral, BID
cefTRIAXone/D5W (cefTRIAXone) 1 gm 50 mL, IV Piggyback, q24H Interval
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro (HumaLOG) 1 unit/0.01 mL (10 mL) (insulin lispro (HumaLOG) Correctional Scale Low) 1-6 Units, SubCutaneous, Before Meals
levETIRAcetam 750 mg Tab (Keppra) 750 mg 1 Tabs, Oral, BID
metoprolol tartrate 25 mg Tab (Metoprolol Tartrate) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (16)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q4H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H

Discharge Summary

labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
lidocaine topical 5% Cream 15 gm (lidocaine 5% topical cream) 1 app, Topical, QID
LORazepam 2 mg/mL inj 1 mL (Ativan) 2 mg 1 mL, IV Push, q2Min Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
morphine 4 mg/mL injection 1 mL (morphine) 2-4 mg, IV Push, q4H
naloxone 0.4 mg/mL Inj 1 mL (naloxone) 0.4 mg 1 mL, IV Push, As Directed
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
polyethylene glycol 3350 Oral Pwdr 17 gm pkt (MiraLax) 17 gm, Oral, Daily
simethicone 80 mg Chew Tab (Mylicon) 80 mg 1 Tabs, Oral, q6H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 1 gm, 50 mL, (UTI), 100 mL/hr, IV Piggyback, q24H Interval	3	03/24/2026 22:58	03/23/2026 23:09

Follow-Up/Discharge Instructions
Follow-up
Follow up with primary care provider, Within 1 to 2 weeks

Post Discharge Care
Discharge Request
• 03/25/26 10:25:00 PDT, Rehabilitation Unit/Facility
• Order Comment:

Pending Results
CBC with Diff ordered on 03/24/2026
Culture Urine ordered on 03/24/2026
Hepatic/Liver Function Panel ordered on 03/24/2026
Magnesium Level ordered on 03/24/2026
Renal Function Panel ordered on 03/24/2026

Results
Discharge Labs
Blood Gases

Ven pH	7.42 (Normal)	03/23/2026 19:57	**Ven pCO2**	35.6 mmHg (Low)	03/23/2026 19:57
Ven pO2	39.4 mmHg (Normal)	03/23/2026 19:57	**Venous pCO2(T)**	35.6 mmHg (N/A)	03/23/2026 19:57
Venous pH(T)	7.42 (N/A)	03/23/2026 19:57	**Venous pO2(T)**	39.4 mmHg (N/A)	03/23/2026 19:57
Venous HCO3	23.0 mmol/L (Normal)	03/23/2026 19:57	**Ven Base Excess**	-1.3 mmol/L (Normal)	03/23/2026 19:57
Ven sO2	75 % (High)	03/23/2026 19:57	**Ven tHb**	10.6 gm/dL (Low)	03/23/2026 19:57
Ven Hct	32 % (Low)	03/23/2026 19:57	**POC Pt Temp**	37.0 DegC (N/A)	03/23/2026 19:57
POC FIO2	21 % (N/A)	03/23/2026 19:57	**POC Sample**	Venous (N/A)	03/23/2026 19:57

Discharge Summary

POC Site	Nurse Collect (N/A)	03/23/2026 19:57	**POC Allen Test**	N/A (N/A)	03/23/2026 19:57
POC Vent Mode	NO VENT (N/A)	03/23/2026 19:57			

Cardiac

Troponin TNIH	13.34 ng/L (Normal)	03/23/2026 19:15			

General Chemistry

Glucose Level	88 mg/dL (Normal)	03/25/2026 01:20	**POC Glucose**	135 mg/dL (High)	03/24/2026 18:03
Capillary Blood Glucose POC Result	135 mg/dL (High)	03/24/2026 18:04	**Sodium**	139 mmol/L (Normal)	03/25/2026 01:20
Ven Na	138 mmol/L (Normal)	03/23/2026 19:57	**Potassium**	4.0 mmol/L (Normal)	03/25/2026 01:20
Ven K	3.4 mmol/L (Low)	03/23/2026 19:57	**Chloride**	108 mmol/L (High)	03/25/2026 01:20
CO2	19 mmol/L (Low)	03/25/2026 01:20	**Anion Gap**	12 mmol/L (Normal)	03/25/2026 01:20
BUN	12 mg/dL (Normal)	03/25/2026 01:20	**Creatinine**	0.82 mg/dL (Normal)	03/25/2026 01:20
BUN/Creat Ratio	15 (Normal)	03/25/2026 01:20	**Calcium**	8.8 mg/dL (Normal)	03/25/2026 01:20
Ven iCa	1.01 mmol/L (Low)	03/23/2026 19:57	**Albumin. Level**	3.7 gm/dL (Normal)	03/25/2026 01:20
TP	5.9 gm/dL (Normal)	03/25/2026 01:20	**A/G Ratio**	1.7 (Normal)	03/25/2026 01:20
T Bili	0.3 mg/dL (Normal)	03/25/2026 01:20	**D Bili**	0.1 mg/dL (Normal)	03/25/2026 01:20
Alk Phos	77 units/L (Normal)	03/25/2026 01:20	**AST**	15 units/L (Normal)	03/25/2026 01:20
ALT	9 units/L (Low)	03/25/2026 01:20	**Mg Lvl**	1.9 mg/dL (Normal)	03/25/2026 01:20
Phosphorus	5.3 mg/dL (High)	03/25/2026 01:20	**Estimated Creatinine Clearance**	46.46 mL/min ()	03/25/2026 03:13
eGFR Cr	72 mL/min/1.73m2 (N/A)	03/25/2026 01:20	**eGFR Pediatric**	Not Reported (N/A)	03/25/2026 01:20
Lactic Acid Lvl	1.4 mmol/L (Normal)	03/24/2026 06:38	**Hgb A1C**	4.8 % (Normal)	03/23/2026 19:15
Iron	25 mcg/dL (Low)	03/23/2026 19:15	**Total Iron Binding Capacity**	124 mcg/dL (Low)	03/23/2026 19:15
Transferrin Saturation	20 % (Normal)	03/23/2026 19:15	**Ferritin Lvl**	116.3 ng/mL (Normal)	03/23/2026 19:15
MAR Glucose Result	91 ()	03/25/2026 09:52			

General Coagulation

PT	12.8 Seconds (High)	03/23/2026 19:15	**INR**	1.2 (High)	03/23/2026 19:15
aPTT	30 Seconds (Normal)	03/23/2026 19:15	**Heparin Anti-Xa (UFH)**	<0.10 Intl_units/mL (Low)	03/23/2026 19:15

General Hematology

WBC	7.90 x10e3/mcL (Normal)	03/25/2026 01:20	**RBC**	3.46 x10e6/mcL (Low)	03/25/2026 01:20
Hgb	9.9 gm/dL (Low)	03/25/2026 01:20	**Hct**	33.4 % (Normal)	03/25/2026 01:20
MCV	96.5 Femtoliters (Normal)	03/25/2026 01:20	**MCH**	28.6 pg (Normal)	03/25/2026 01:20

Discharge Summary

MCHC	29.6 gm/dL (Low)	03/25/2026 01:20	**RDW-SD**	50.3 Femtoliters (High)	03/25/2026 01:20
RDW-CV	14.2 % (Normal)	03/25/2026 01:20	**Plt**	236 x10e3/mcL (Normal)	03/25/2026 01:20
MPV	9.4 Femtoliters (Normal)	03/25/2026 01:20	**Imm Plt Fract**	1.0 % (Low)	03/24/2026 06:40
Neut % Auto	78.9 % (N/A)	03/25/2026 01:20	**Lymph % Auto**	11.5 % (N/A)	03/25/2026 01:20
Mono % Auto	7.2 % (N/A)	03/25/2026 01:20	**Eos % Auto**	1.4 % (N/A)	03/25/2026 01:20
Baso % Auto	0.5 % (N/A)	03/25/2026 01:20	**Immature Grans %**	0.5 % (Normal)	03/25/2026 01:20
Neut # Auto	6.23 x10e3/mcL (Normal)	03/25/2026 01:20	**Lymph # Auto**	0.91 x10e3/mcL (Normal)	03/25/2026 01:20
Mono # Auto	0.57 x10e3/mcL (Normal)	03/25/2026 01:20	**Eos # Auto**	0.11 x10e3/mcL (Normal)	03/25/2026 01:20
Baso # Auto	0.04 x10e3/mcL (Normal)	03/25/2026 01:20	**Immature Grans # Auto**	0.04 x10e3/mcL (Normal)	03/25/2026 01:20

Other Chemistry

Beta Hydroxybutyrate Lvl	5.9 mg/dL (High)	03/23/2026 20:33			

Other Microbiology

MRSA by PCR	Detected (Abnormal)	03/24/2026 09:47			

Thyroid

T3 Free	2.33 pg/mL (Normal)	03/23/2026 19:15	**T4 Free**	0.94 ng/dL (Normal)	03/23/2026 19:15
TSH Ultrasensitive	2.821 mc Intl units/mL (Normal)	03/23/2026 19:15			

UA Macroscopic

Ur Color	Yellow (Normal)	03/24/2026 09:47	**Ur Clarity**	Cloudy (Abnormal)	03/24/2026 09:47
UR Glucose	Negative (Normal)	03/24/2026 09:47	**Ur Bili**	Negative (Normal)	03/24/2026 09:47
Ur Ketones	Negative (Normal)	03/24/2026 09:47	**Ur Spec Grav**	1.010 (Normal)	03/24/2026 09:47
Ur Blood	2+ (Abnormal)	03/24/2026 09:47	**Ur pH**	6.0 (Normal)	03/24/2026 09:47
Ur Protein	1+ (Abnormal)	03/24/2026 09:47	**Ur Urobilinogen**	<2.0 mg/dL (Normal)	03/24/2026 09:47
Ur Nitrite	Positive (Abnormal)	03/24/2026 09:47	**Ur Leuk Est**	2+ (Abnormal)	03/24/2026 09:47
UA Culture?	Yes (N/A)	03/24/2026 09:47	**Signs/Symptoms of UTI?**	Low Ur Symptom (N/A)	03/24/2026 09:47
UTI Other:	abdominal pain (N/A)	03/23/2026 14:58			

UA Microscopic

UA WBC Inst	29 /HPF (High)	03/24/2026 09:47	**UA RBC Inst**	37 /HPF (High)	03/24/2026 09:47
Ur Bacteria	1+ /HPF (Abnormal)	03/24/2026 09:47	**UA Squam Epi Inst**	1 /HPF (Normal)	03/24/2026 09:47
Ur Mucous	Rare /LPF (Abnormal)	03/23/2026 14:58	**Ur Hyal Cast**	1 /LPF (Normal)	03/24/2026 09:47

Discharge Summary

Urine Chemistry

Ur Sodium	34 mmol/L (N/A)	03/24/2026 09:47	**Ur Protein/Creatinine**	1.4 (High)	03/24/2026 09:47
Ur Total Protein	63.3 mg/dL (High)	03/24/2026 09:47	**Ur Creat**	44.9 mg/dL (N/A)	03/24/2026 09:47
Ur Creatinine	44.3 mg/dL (N/A)	03/24/2026 09:47	**U Microalbumin**	7.5 mg/dL (N/A)	03/24/2026 09:47
Microalb/Crea Ratio	164.9 mg/gm (N/A)	03/24/2026 09:47			

Microbiology

Culture Urine **In-Lab**

Source: U CleanCatch Body Site:
Collected Dt/Tm: 03/24/2026 09:47 Last Updated Dt/Tm: 03/24/2026 10:50

Allergies
Allergies (Active and Proposed Allergies Only)
Adhesive Bandage (Severity: Moderate, Onset: Unknown)
Reactions: Itching of skin

Objective

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.5	03/25/2026 07:46	36.8	03/24/2026 19:58	37.1	03/24/2026 23:10
Apical Heart Rate	83	03/25/2026 09:36	72	03/24/2026 20:28	83	03/25/2026 09:36
Resp Rate	18	03/25/2026 07:46	17	03/25/2026 04:37	20	03/24/2026 12:38
SBP	133	03/25/2026 07:46	124	03/24/2026 18:00	**H** 173	03/24/2026 11:00
DBP	64	03/25/2026 07:46	64	03/25/2026 07:46	**H** 82	03/24/2026 14:00
MAP	91	03/25/2026 07:46	91	03/24/2026 18:00	112	03/24/2026 11:00
SpO2	94	03/25/2026 07:46	94	03/25/2026 07:46	100	03/24/2026 11:00
O2 Therapy	Room air		Room air		Room air	

Intake and Output (current encounter)

	03/23 12:09 03/25 10:31	**03/25 07:00 03/25 10:31**	**03/24 07:00 03/25 06:59**	**03/23 07:00 03/24 06:59**	**03/22 07:00 03/23 06:59**
Intake	700.0	0	500.0	200	0
Output	0	0	0	0	0
Net Total	700.0	0	500.0	200	0
Urine Count	4	2	2	0	0

Discharge Summary

Physical Exam:
General: 81yo Female laying in bed in no acute distress
Mental Status: Alert and oriented to person, place and time. Normal affect.
HEENT: Normocephalic, Atraumatic. PERRL. EOMI. Normal dentition. Oropharynx clear, mucous membranes moist. Ears and nose without lesions or deformities. Full range of motion. Trachea midline.
Respiratory: CTA bilaterally, no crackles, rales, wheezes, rhonchi.
Cardiovascular: Regular rate and rhythm, no murmurs, rubs or gallops, normal S1, S2, no peripheral edema.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Bowel sounds normal.
Genitourinary: No CVA tenderness, No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Hospital Course
81-year-old female with past medical history of abnormal uterine bleeding and a fib on anticoagulation, was scheduled for hysterectomy was found to have a low potassium at her PCP prior to scheduled surgery was brought to Valley Hospital Medical Center on 3/23/2026 to treat hypokalemia. Patient surgery was rescheduled for the following week. Upon admission patient hemodynamically stable, saturating well on room air. Potassium elevated 2.4 EKG showed atrial fibrillation. Patient is not on anticoagulation for her A-fib due to severe GI bleeding previously. Patient required multiple doses of IV and oral potassium to replete levels reading of 4.0. PT OT evaluated the patient and determined that patient would benefit from acute rehab following discharge to improve functional deficits. Recommended the patient follow-up with her PCP regarding hypokalemia.

On discharge, patient has stable vitals, labs within normal limits, and acceptable physical exam. Patient denies any symptoms at this time. Patient was advised to follow-up with PCP within the next 1-2 weeks. Patient is to return to ED if symptoms worsen or if there are any questions or concerns. All questions and concerns were answered and addressed at bedside. Patient is currently alert and oriented to person, place, time, situation. Risk, benefits and alternatives conveyed. Patient has verbalized understanding of diagnoses, management, treatment, and follow-up instructions. Patient is safe for discharge at this time.

Greater than 30 minutes were spent in the examination, evaluation, counseling, and review of medications and discharge plan for this patient.

This patient will be discussed with the attending physician. Further recommendations to follow.

Trevor Orr, MD
PGY-3 Family Medicine
Valley Hospital Medical Center

Electronically Signed By: Orr, Trevor MD
On: 03.25.2026 10:32 PDT

Electronically Signed On: 03.25.2026 11:42 PDT
Techathaveewat, Worawith DO

VHM- Valley Hospital Medical Center
620 Shadow Lane
Las Vegas, NV 89106-4194
(702) 388-4000

Patient Information

Patient Name: COLE, CAROL GASTON
Home Address: 4720 FAIRFAX AVE
LAS VEGAS, NV 891201711
Home: (702) 451-0591
Employer Name: Retired
Employer Phone:
Retire Date: 12/01/1996

Sex: Female **Language:** English
DOB: 12/28/1944 **Race:** White
Age: 81 Years **SSN:** XXX-XX-8550
Religion: Other
Marital Status: Divorced
Contact: Phone Calls and Visitor
EMail: No Email Address

Guarantor Information

Guarantor Name: COLE, CAROL GASTON
Patient's Reltn: Self
Billing Address: 4720 FAIRFAX AVE
LAS VEGAS, NV 891201711
Billing Phone: (702) 451-0591

Sex: Female **DOB:** 12/28/1944
Employer Name: Retired
Employer Phone:

Contact Information

Emergency Contact
Contact Name: MINDY ROMAN
Patient's Reltn: Friend
Sex: Female
Preferred Phone: (702) 218-5292

Next of Kin
Contact Name:
Patient's Reltn:
Sex:
Preferred Phone:

Primary Insurance

Subscriber Name: COLE, CAROL GASTON
Patient's Reltn: Self
Sex: Female
DOB: 12/28/1944

Employer Name: Retired
Employer Phone:
Financial Class: Medicare
Group Name:

Insurance Name: Medicare Part A Only - VHS
Claim Address: MEDICARE PART A
MECHANICSBURG, PA 17055
Insurance Phone: (877) 235-8073
Policy Number: 7PP0DC8GU25
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Secondary Insurance

Subscriber Name: COLE, CAROL GASTON
Patient's Reltn: Self
Sex: Female
DOB: 12/28/1944

Employer Name: Retired
Employer Phone:
Financial Class: Medicaid
Group Name:

Insurance Name: Medicaid Secondary to Medicare -
Claim Address: MEDICAID 2ND MCARE
Reno, NV 89520
Insurance Phone: (877) 638-3472
Policy Number: 51873700001
Group Number:
Authorization Number:
Authorization Phone:
Auth Req:
Authorization Contact:

Encounter Information

Reg Dt/Tm: 03/23/2026 12:09 PDT
Est Dt of Arrival: 04/01/2026 07:00 PDT
Inpt Adm Dt/Tm: 03/25/2026 10:07 PDT
Disch Dt/Tm:
Observ Dt/Tm:
VIP Indicator:
Admit Reason: HYPOKALEMIA
Accident:
Accident Dt/Tm:

Patient Type: Inpatient
Medical Service: Medicine
Location: VHM 3N
Room/Bed: 316 / 01
Est Responsibility: 0.00

Admit Type: Emergency
Admit Source: Emergency Department
Reg Clerk: Akabutu, Frank
Admitting: Milne, C Dean DO
Attending Techathaveewat DO, Worawith
PCP: Provider Unlisted
Referring: Provider Unlisted
Last Visit Date: 03/23/2026

COLE, CAROL GASTON
Female / 81 Years

MEDICAL RECORD

EL0012

CCPS_REG_XR_FACESHEET Printed By: USER, XR DEFAULT on 03/25/2026 17:02to printer cr_facesheet_12002
Registration last updated by: Cook, Lanisha on 03/25/2026 10:18

Patient Identification

1101309621-63890719
COLE, CAROL GASTON
12/28/1944 81 Ye F
ADM/REG DT: 03/25/2026 10:07

Patient: COLE, CAROL GASTON
MRN: LVMG8010101631; DSH5144223; VHM63890719
DOB/Sex: 12/28/1944 / Female

Admit: 3/23/2026
Disch: Disch Time:
FIN: VHM1101309621
Readmit Risk: 34 - LOW 03/25/26 13:45
Attending: Techathaveewat DO,Worawith

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 3/25/2026 10:11 PDT			
Signed Date/Time: 3/25/2026 10:11 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 3/25/2026 10:11 PDT		End-state Reason:	
Ordering Physician: Orr MD,Trevor		Consulting Physician:	
Entered By: Orr MD,Trevor on 3/25/2026 10:11 PDT			
Order Details: 3/25/26 10:11:00 AM PDT, Routine, Reason for Consult: D/C to Home w Home Health, IV: Yes, O2: No			
Order Comment:			

Orders - Admission/Discharge (Curaspan)

Order: **Update Emergency Contact**			
Order Date/Time: 3/25/2026 15:10 PDT			
Signed Date/Time: 3/25/2026 15:10 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 15:19 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 15:10 PDT			
Order Details: 3/25/26 3:10:47 PM PDT			
Order Comment: Contact Name: ROMAN, MINDY Relationship: daughter Phone: (702) 218-5292			
Action Type: Complete	Action Date/Time: 3/25/2026 15:19 PDT	Action Personnel: Phelps,Shanae	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:	
Order Details: 03/25/26 15:10:47 PDT			
Review Information: Doctor Cosign: Not Required			
Order Comment:			
Action Type: Order	Action Date/Time: 3/25/2026 15:10 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 03/25/26 15:10:47 PDT			
Review Information: Doctor Cosign: Not Required			
Order Comment: Contact Name: ROMAN, MINDY Relationship: daughter Phone: (702) 218-5292			

Orders - Admission/Discharge (Curaspan)

Order: **Status Change**			
Order Date/Time: 3/25/2026 10:07 PDT			
Signed Date/Time: 3/25/2026 10:07 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:17 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:07 PDT			
Order Details: 3/25/26 10:07:38 AM PDT, Admit as Inpatient, Acute Med/Surg with Telemetry			
Order Comment: Status: Admit as Inpatient. Diagnosis: Hypokalemia. Ordered secondary to modification of Status on the Admission/Observation/Transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/25/2026 10:17 PDT	Action Personnel: Cook,Lanisha
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/25/26 10:07:38 PDT, Admit as Inpatient, Acute Med/Surg with Telemetry		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 10:07 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:07:38 PDT, Admit as Inpatient, Acute Med/Surg with Telemetry		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 10:10 PDT Doctor Cosign: Not Required		
Order Comment: Status: Admit as Inpatient. Diagnosis: Hypokalemia. Ordered secondary to modification of Status on the Admission/Observation/Transfer order. 1.00		

Order: **Discharge Request Task Order**			
Order Date/Time: 3/25/2026 10:26 PDT			
Signed Date/Time: 3/25/2026 10:26 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:26 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:26 PDT			
Order Details: 3/25/26 10:26:02 AM PDT, Rehabilitation Unit/Facility			
Order Comment:			

Action Type: Order	Action Date/Time: 3/25/2026 10:26 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:26:02 PDT, Rehabilitation Unit/Facility		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Patient:	COLE, CAROL GASTON		Admit: 3/23/2026
MRN:	LVMG8010101631; DSH5144223; VHM63890719		Disch:
DOB/Sex:	12/28/1944 / Female	FIN:	VHM1101309621
Attending:	Techathaveewat DO,Worawith		

Orders - Admission/Discharge (Curaspan)

Order: **Discharge Request Task Order**			
Order Date/Time: 3/25/2026 16:34 PDT			
Signed Date/Time: 3/25/2026 16:34 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 16:34 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 16:34 PDT			
Order Details: 3/25/26 4:34:01 PM PDT, Home with Home Health			
Order Comment:			

Action Type: Order	Action Date/Time: 3/25/2026 16:34 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 16:34:01 PDT, Home with Home Health		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Order: **Discharge Request**			
Order Date/Time: 3/25/2026 10:25 PDT			
Signed Date/Time: 3/25/2026 10:25 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:25 PDT		End-state Reason:	
Ordering Physician: Orr MD,Trevor		Consulting Physician:	
Entered By: Orr MD,Trevor on 3/25/2026 10:25 PDT			
Order Details: 3/25/26 10:25:00 AM PDT, Home with Home Health			
Order Comment:			

Action Type: Modify	Action Date/Time: 3/25/2026 16:33 PDT	Action Personnel: Orr MD,Trevor
Responsible Provider/Electronically Signed by: Orr MD,Trevor	Supervising Provider:	Communication Type: Written
Order Details: 03/25/26 10:25:00 PDT, Home with Home Health		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 16:39 PDT Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 10:26 PDT	Action Personnel: Orr MD,Trevor
Responsible Provider/Electronically Signed by: Orr MD,Trevor	Supervising Provider:	Communication Type: Written
Order Details: 03/25/26 10:25:00 PDT, Rehabilitation Unit/Facility		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 10:29 PDT Doctor Cosign: Not Required		
Order Comment:		

Patient: COLE, CAROL GASTON Admit: 3/23/2026
MRN: LVMG8010101631; DSH5144223; VHM63890719 Disch:
DOB/Sex: 12/28/1944 / Female FIN: VHM1101309621
Attending: Techathaveewat DO,Worawith

Orders - Admission/Discharge (Curaspan)

Order: **Change Attending Physician**			
Order Date/Time: 3/25/2026 10:07 PDT			
Signed Date/Time: 3/25/2026 10:07 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 10:18 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:07 PDT			
Order Details: 3/25/26 10:07:38 AM PDT, Attending: Techathaveewat DO, Worawith			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 3/25/2026 10:18 PDT	Action Personnel: Cook,Lanisha
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 03/25/26 10:07:38 PDT, Attending: Techathaveewat DO, Worawith		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 3/25/2026 10:07 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 03/25/26 10:07:38 PDT, Attending: Techathaveewat DO, Worawith		
Review Information: Nurse Review: Electronically Signed, Heimerman RN,Paula on 3/25/2026 10:10 PDT Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Patient:	COLE, CAROL GASTON		Admit: 3/23/2026
MRN:	LVMG8010101631; DSH5144223; VHM63890719		Disch:
DOB/Sex:	12/28/1944 / Female	FIN:	VHM1101309621
Attending:	Techathaveewat DO,Worawith		

Orders - Admission/Discharge (Curaspan)

Order: **Case Management Discharge Notification**			
Order Date/Time: 3/25/2026 10:26 PDT			
Signed Date/Time: 3/25/2026 10:26 PDT			
Order Status: Canceled	Department Status: Canceled	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 16:34 PDT		End-state Reason: Order Modified	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 10:26 PDT			
Order Details: 3/25/26 10:26:02 AM PDT			
Order Comment: Rehabilitation Unit/Facility			
Action Type: Cancel	Action Date/Time: 3/25/2026 16:34 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 03/25/26 10:26:02 PDT			
Review Information: Doctor Cosign: Not Required			
Order Comment:			
Action Type: Order	Action Date/Time: 3/25/2026 10:26 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 03/25/26 10:26:02 PDT			
Review Information: Doctor Cosign: Not Required			
Order Comment: Rehabilitation Unit/Facility			

Order: **Case Management Discharge Notification**			
Order Date/Time: 3/25/2026 16:34 PDT			
Signed Date/Time: 3/25/2026 16:34 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 3/25/2026 16:34 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 3/25/2026 16:34 PDT			
Order Details: 3/25/26 4:34:00 PM PDT			
Order Comment: Home with Home Health			
Action Type: Order	Action Date/Time: 3/25/2026 16:34 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 03/25/26 16:34:00 PDT			
Review Information: Doctor Cosign: Not Required			
Order Comment: Home with Home Health			

Medication Administration Summary

Medications

Administration Date/Time: 3/25/2026 12:03 PDT
Medication Name: **levETIRAcetam (Keppra)**
Ingredients: leve750Tab 750 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 3/25/2026 11:30 PDT
Medication Name: **insulin lispro (insulin lispro (HumaLOG) Correctional Scale Low)**
Admin Details: (Not Done) Not Appropriate at this Time
MAR Glucose Result, insulin lispro

Physical Therapy Forms

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	3/25/2026 15:10 PDT
RESULT STATUS:	Modified
PERFORMED INFORMATION:	Garcia,Jonathan (3/25/2026 16:13 PDT)
SIGNED INFORMATION:	Garcia,Jonathan (3/25/2026 16:36 PDT); Garcia,Jonathan (3/25/2026 16:13 PDT)

PT Evaluation/Reevaluation Entered On: 3/25/2026 16:28 PDT
Performed On: 3/25/2026 15:10 PDT by Garcia, Jonathan

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 03/25/26 13:50:00 PDT, Routine, Stop date 03/25/26 13:50:00 PDT, Stretcher, Yes, No
Precautions RTF : Active Orders:
» Precautions, 03/23/26 20:06:00 PDT, Routine, Precautions: Seizure, Ordered
» Oxygen Therapy, 03/23/26 18:39:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to maintain SpO2 or SaO2 92-98%, Ordered
» Precautions, 03/23/26 18:39:00 PDT, Routine, Precautions: Fall, Ordered

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate, Calm, Cooperative
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Garcia, Jonathan - 3/25/2026 16:13 PDT

Problem List

(As Of: 3/25/2026 16:28:02 PDT)

Problems(Active)

Physical Therapy Forms

Acute CHF (SNOMED CT :18472010)	Name of Problem: Acute CHF ; Recorder: Hoppler RN, Sue; Confirmation: Confirmed ; Classification: Medical ; Code: 18472010 ; Contributor System: PowerChart ; Last Updated: 6/9/2015 8:28 PDT ; Life Cycle Date: 6/9/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
Acute pain (SNOMED CT :410499012)	Name of Problem: Acute pain ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 410499012 ; Last Updated: 6/12/2015 4:56 PDT ; Life Cycle Date: 6/12/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/12/2015 4:56 - SYSTEM This problem was added by Discern Expert.
Arthritis (SNOMED CT :7278014)	Name of Problem: Arthritis ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 7278014 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 10:43 PDT ; Life Cycle Date: 3/18/2026 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
At risk for falls (SNOMED CT :208683018)	Name of Problem: At risk for falls ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 208683018 ; Last Updated: 6/9/2015 20:42 PDT ; Life Cycle Date: 6/9/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/9/2015 20:42 - SYSTEM This problem was added by Discern Expert.
At risk of pressure sore (SNOMED CT :424268015)	Name of Problem: At risk of pressure sore ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 424268015 ; Last Updated: 6/9/2015 20:36 PDT ; Life Cycle Date: 6/9/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/9/2015 20:36 - SYSTEM This problem was added by Discern Expert.
Bowel dysfunction (SNOMED CT :353134013)	Name of Problem: Bowel dysfunction ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 353134013 ; Last Updated: 6/11/2015 8:24 PDT ; Life Cycle Date: 6/11/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/9/2015 20:36 - SYSTEM Problem added automatically by system based on

Physical Therapy Forms

	documentation of abnormal bowel sounds, abnormal GI symptoms, or passing flatus = no.
HTN (hypertension) (SNOMED CT :1215744012)	Name of Problem: HTN (hypertension) ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 1215744012 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 10:43 PDT ; Life Cycle Date: 3/18/2026 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
Ineffective airway clearance (SNOMED CT :112516014)	Name of Problem: Ineffective airway clearance ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Nursing ; Code: 112516014 ; Last Updated: 6/11/2015 19:49 PDT ; Life Cycle Date: 6/11/2015 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 6/11/2015 19:49 - SYSTEM Problem added automatically by system based on documentation of inability to clear secretions, abnormal lung sounds, shortness of breath or O2Sat less than 92%.
MRSA (SNOMED CT :176116017)	Name of Problem: MRSA ; Onset Date: 3/24/2026 ; Recorder: SYSTEM; Confirmation: Confirmed ; Classification: Medical ; Code: 176116017 ; Contributor System: PowerChart ; Last Updated: 3/24/2026 12:47 PDT ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments: 3/18/2026 14:11 - SYSTEM Problem added by Discern Expert based on Positive MRSA test result 3/19/2026 6:04 - Walsh RN, Timothy F PCR (nares) 3/18/26 3/24/2026 12:47 - Thapar RN, Anusha PCR (nares) 3/24/26
Petit mal seizure status (SNOMED CT :4636990012)	Name of Problem: Petit mal seizure status ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 4636990012 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 10:43 PDT ; Life Cycle Date: 3/18/2026 ; Life Cycle Status: Active ; Vocabulary: SNOMED CT
Transfusion history (SNOMED CT :2986959016)	Name of Problem: Transfusion history ; Recorder: Norwood RN, Yvonne; Confirmation: Confirmed ; Classification: Medical ; Code: 2986959016 ; Contributor System: PowerChart ; Last Updated: 3/18/2026 11:08 PDT ; Life Cycle Status: Active ; Vocabulary: SNOMED CT ; Comments:

Physical Therapy Forms

3/18/2026 11:08 - Norwood RN, Yvonne
YEAR: 2015.

Diagnoses(Active)

Abnormal laboratory findings	Date: 3/23/2026 ; Diagnosis Type: Reason For Visit ; Confirmation: Complaint of ; Clinical Dx: Abnormal laboratory findings ; Classification: Nursing ; Clinical Service: Non-Specified ; Code: PNED ; Probability: 0 ; Diagnosis Code: 041FFDE2-A217-4FDF-B142-F003962BF651
AKI (acute kidney injury)	Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: AKI (acute kidney injury) ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: N17.9
Hypokalemia	Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: Hypokalemia ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: E87.6
Hypokalemia	Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: Hypokalemia ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: E87.6
Hypomagnesemia	Date: 3/23/2026 ; Diagnosis Type: Discharge ; Confirmation: Confirmed ; Clinical Dx: Hypomagnesemia ; Classification: Medical ; Clinical Service: Non-Specified ; Code: ICD-10-CM ; Probability: 0 ; Diagnosis Code: E83.42

Home Environment
Living Environment : Home Environment
Devices/Equipment at Home: Shower chair, Walker, Wheelchair Performed By: Paine RN, Danielle 03/25/2026
Living Situation: Home independently Performed By: Paine RN, Danielle 03/25/2026
Professional Skilled Services: Other: PCA Performed By: Paine RN, Danielle 03/25/2026
Living Situation : Home independently
Lives With : Child(ren)
Lives In : Multilevel home

Garcia, Jonathan - 3/25/2026 16:13 PDT

Location Bed
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Garcia, Jonathan - 3/25/2026 16:13 PDT

Patient's Responsibilities Rehab : Personal ADL

Garcia, Jonathan - 3/25/2026 16:13 PDT

Home Environment II
Living Environment : Home Environment
Devices/Equipment at Home: Shower chair, Walker, Wheelchair Performed By: Paine RN, Danielle 03/25/2026
Living Situation: Home independently Performed By: Paine RN, Danielle 03/25/2026
Professional Skilled Services: Other: PCA Performed By: Paine RN, Danielle 03/25/2026

Physical Therapy Forms

Home Equipment Rehab : Elevated toilet seat, 4-Wheel walker, Front-wheel walker (FWW), Shower commode chair
Garcia, Jonathan - 3/25/2026 16:13 PDT

Prior Functional Status Grid
ADL : Independent
Mobility : Independent
Instrumental ADL : Independent
Cognitive-Communication Skills : Independent
Garcia, Jonathan - 3/25/2026 16:13 PDT

LE Range/Strength
LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits
Garcia, Jonathan - 3/25/2026 16:13 PDT

Lt Lower Extremity Strength : Within functional limits
Rt Lower Extremity Strength : Within functional limits
Garcia, Jonathan - 3/25/2026 16:13 PDT

Balance
Kansas University Balance Scale : Yes
General Balance : Both
General Balance Assessment : Fair
Sitting Balance Score : 4
Standing Balance Score : 3+
Garcia, Jonathan - 3/25/2026 16:13 PDT

Mobility
Mobility Grid
Roll Left : Rehab Modified independence
Roll Right : Rehab Modified independence
Supine to Sit : Rehab Modified independence
Sit to Supine : Rehab Modified independence
Transfer Sit to Stand : Rehab Modified independence
Transfer Stand to Sit : Rehab Modified independence
Transfer Bed to and From Chair : Rehab Modified independence
Garcia, Jonathan - 3/25/2026 16:13 PDT

Ambulation Level : Close supervision
Device : Gait belt, Front-wheel walker (FWW)
Ambulation Distance : 250 ft
Gait Deviations Left Lower Extremity : Decreased stride length
Gait Deviations Right Lower Extremity : Decreased stride length
Garcia, Jonathan - 3/25/2026 16:13 PDT

Orthostatics
Systolic/Diastolic Sitting BP : 118 mmHg
Systolic/Diastolic Sitting BP : 63 mmHg
Systolic/Diastolic Standing BP : 124 mmHg
Systolic/Diastolic Standing BP : 64 mmHg
Patient Response to Orthostatics : pt denies dizziness/lightheadedness
Garcia, Jonathan - 3/25/2026 16:13 PDT

Assessment

Physical Therapy Forms

PT Impairments or Limitations : Ambulation deficits, Balance deficits
Barriers to Safe Discharge PT : Limited family support, Medical diagnosis, Severity of deficits, Time since onset
Garcia, Jonathan - 3/25/2026 16:13 PDT

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident
Garcia, Jonathan - 3/25/2026 16:13 PDT

Physical Therapy Education Grid
Ambulation with Standard Walker : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Bed Mobility : Demonstrates
Bed Positioning : Demonstrates
Bed to Chair Transfers : Demonstrates
Body Mechanics : Demonstrates
Physical Therapy Plan of Care : Demonstrates
Sit to Stand Transfers : Demonstrates
Garcia, Jonathan - 3/25/2026 16:13 PDT

Long Term Goals
PT Patient,Caregiver Goal : Pt to get well and go home with family
Garcia, Jonathan - 3/25/2026 16:13 PDT

Short Term Goals
Other PT Goals Grid

	Goal #1
Goal :	Pt to ambulate >150ft Mod I LRAD
Status :	Initial
Timeframe To Complete :	LOS
	Garcia, Jonathan - 3/25/2026 16:13 PDT

Plan
PT Anticipated Treatments, Needs : Balance training, Basic activities of daily living, Bed mobility training, Equipment training, Gait training, Patient education, Posture/Body mechanics training, Stair training, Therapeutic exercises, Transfer training
PT Frequency Rehab : Other: 1+/wk
PT Duration Rehab : 1-2 weeks
PT Intensity Per Day : Other: 1+/wk
Rehabilitation Potential : Good
Post-Acute Discharge Therapy Recommendation PT : Continued intermittent therapy to address functional deficits
IDR PT Recommend/Goal : Pt to d/c home with family and ongoing skilled therapy
PT Plan/Goals Established w Pt/Caregiver : Yes
Garcia, Jonathan - 3/25/2026 16:13 PDT

Additional Information

Physical Therapy Forms

Additional Information PT : THERAPY SESSION TIME: 53 min
RN CLEARED FOR THERAPY: yes
PAIN LOCATION AND LEVEL: n/a currently
ORIENTATION: AOx4
PLOF: Mod IND

Pt orders received for PT eval, chart reviewed, cleared by RN. Pt presents with hypokalemia; see report for details. Pt was received supine in bed, pleasant and agreeable to PT. Gait belt donned for safety. Pt completed mobility per grid, no LOB, no fall. Pt Mod I for bed mobility, Mod I STS/transfer and SPV ambulation approx 250ft FWW. Pt declined further mobility. PT reviewed walker management and upright posture during mobility, VC for attention to walker placement. Pt would currently benefit from skilled PT during admission to decrease risk for falls and maintain IND. Pt may benefit from skilled PT post d/c to improve upon functional deficits. Pt reports she has assistance at home and equipment. PT rec pt d/c home with ongoing skilled therapy, when medically cleared by physician. Pt was left sitting in chair, per pt request, all needs met, RN updated. Ext time required to complete tasks, rest breaks, slow-steady movements, education. pt declined toileting and hygiene/ADL at this time. chair alarm on

BARRIERS TO SAFE DC/IMPAIRMENTS AND LIMITATIONS: PMH
THERAPY RECOMMENDATION: PT rec pt d/c home with PT, when medically cleared by physician.
Environmental Safety Implemented : Bed in low position, Call device within reach, Lock wheeled equipment, Personal items within reach

Garcia, Jonathan - 3/25/2026 16:13 PDT

PT Non Timed Modalities 2026
PT Treatment Start Time : 3/25/2026 15:10 PDT
PT Treatment End Time : 3/25/2026 16:03 PDT

Garcia, Jonathan - 3/25/2026 16:13 PDT

~~PT Eval Moderate Complexity Total Number Min : 1 [IN ERROR]~~

~~{ [1] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

~~PT Total Non Timed : 1 [IN ERROR]~~

Garcia, Jonathan - 3/25/2026 16:36 PDT

~~{ [1] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

PT Timed Modalities 2026
PT Timed Modality Treatment Start Time : 3/25/2026 15:10 PDT
PT Timed Modality Treatment End Time : 3/25/2026 16:03 PDT
Rehab Patient : No

Garcia, Jonathan - 3/25/2026 16:13 PDT

PT Timed Min Total : 53

~~{ [38] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

PT Timed Unit Total : 4

~~{ [3] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

PT THERAPEUTIC ACT 15MIN : 30

~~{ [15] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

Activity Therapeutic Num of Units : 2

Garcia, Jonathan - 3/25/2026 16:36 PDT

~~{ [1] - previously charted by Garcia, Jonathan at 3/25/2026 16:13 PDT};~~

HOME/FAMILY INSTRUCT 15MIN : 8
Home/Family Instruct Num of Units : 1
PT GAIT TRAINING 15MIN : 15
Gait Training Num of Units : 1

Garcia, Jonathan - 3/25/2026 16:13 PDT

Discharge Summary

DOCUMENT NAME:	Discharge Summary
SERVICE DATE/TIME:	3/25/2026 10:31 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Orr MD,Trevor (3/25/2026 10:32 PDT)
SIGN INFORMATION:	Techathaveewat DO,Worawith (3/25/2026 11:42 PDT); Orr MD,Trevor (3/25/2026 10:32 PDT)

Patient Information
Attending Physician: Techathaveewat DO, Worawith
Admitting Diagnosis: Hypokalemia
Discharge Location: VHM 3N
Primary Care Physician:
Inpatient Admit Date/Time: 03/25/2026 10:07

Discharge Diagnosis
Hypokalemia (E87.6)
AKI (acute kidney injury) (N17.9)
Hypomagnesemia (E83.42)
Hypokalemia (E87.6)

Discharge Medications
carBAMazepine (carBAMazepine 200 mg oral tablet) 200 Milligram By Mouth 2 Times a Day Indication: SEIZURES
clopidogrel (clopidogrel 75 mg oral tablet) 75 Milligram By Mouth Daily
ipratropium nasal (ipratropium 21 mcg/inh (0.03%) nasal spray) 21 MCG / INH Nasal Inhalation 2 Times a Day
levETIRAcetam (levETIRAcetam 750 mg oral tablet) 750 Milligram By Mouth 2 Times a Day Indication: SEIZURES
metoprolol (metoprolol tartrate 25 mg oral tablet) 25 Milligram By Mouth 2 Times a Day
olmesartan (olmesartan 40 mg oral tablet) 40 Milligram By Mouth Daily

Inpatient Medications
Medications (23) Active
Scheduled: (7)
carBAMazepine 200 mg Tab (carBAMazepine 200 mg oral tablet) 200 mg 1 Tabs, Oral, BID
cefTRIAXone/D5W (cefTRIAXone) 1 gm 50 mL, IV Piggyback, q24H Interval
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro (HumaLOG) 1 unit/0.01 mL (10 mL) (insulin lispro (HumaLOG) Correctional Scale Low) 1-6 Units, SubCutaneous, Before Meals
levETIRAcetam 750 mg Tab (Keppra) 750 mg 1 Tabs, Oral, BID
metoprolol tartrate 25 mg Tab (Metoprolol Tartrate) 25 mg 1 Tabs, Oral, BID
Continuous: (0)
PRN: (16)
acetaminophen 325 mg Tab (Tylenol) 650 mg 2 Tabs, Oral, q4H
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q4H
calcium carbonate 500 mg Chew Tab (Tums 500) 500 mg 1 Tabs, Oral, BID
Dextrose 50% Injection 50 mL (Dextrose 50% in Water Injection) 25 gm 50 mL, IV Push, q1H Interval
glucagon recombinant 1 mg vial (glucagon) 1 mg 1 mL, IntraMuscular, q4H
glucose 15 gm in 37.5 gm Gel (40%) (Glutose) 15 gm 1 Each, Oral, q1H Interval
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H

Discharge Summary

labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H
lidocaine topical 5% Cream 15 gm (lidocaine 5% topical cream) 1 app, Topical, QID
LORazepam 2 mg/mL inj 1 mL (Ativan) 2 mg 1 mL, IV Push, q2Min Interval
melatonin 5 mg Tab (melatonin 5 mg oral tablet) 5 mg 1 Tabs, Oral, qHS
morphine 4 mg/mL injection 1 mL (morphine) 2-4 mg, IV Push, q4H
naloxone 0.4 mg/mL Inj 1 mL (naloxone) 0.4 mg 1 mL, IV Push, As Directed
ondansetron 4 mg/2 mL vial (Zofran) 4 mg 2 mL, IV Push, q8H
polyethylene glycol 3350 Oral Pwdr 17 gm pkt (MiraLax) 17 gm, Oral, Daily
simethicone 80 mg Chew Tab (Mylicon) 80 mg 1 Tabs, Oral, q6H

72 hour Antibiotic History

Active Antibiotics	Day of Therapy:	Last Administered	First Administered
cefTRIAXone 1 gm, 50 mL, (UTI), 100 mL/hr, IV Piggyback, q24H Interval	3	03/24/2026 22:58	03/23/2026 23:09

Follow-Up/Discharge Instructions
Follow-up
Follow up with primary care provider, Within 1 to 2 weeks

Post Discharge Care
Discharge Request
• 03/25/26 10:25:00 PDT, Rehabilitation Unit/Facility
• Order Comment:

Pending Results
CBC with Diff ordered on 03/24/2026
Culture Urine ordered on 03/24/2026
Hepatic/Liver Function Panel ordered on 03/24/2026
Magnesium Level ordered on 03/24/2026
Renal Function Panel ordered on 03/24/2026

Results
Discharge Labs
Blood Gases

Ven pH	7.42 (Normal)	03/23/2026 19:57	**Ven pCO2**	35.6 mmHg (Low)	03/23/2026 19:57
Ven pO2	39.4 mmHg (Normal)	03/23/2026 19:57	**Venous pCO2(T)**	35.6 mmHg (N/A)	03/23/2026 19:57
Venous pH(T)	7.42 (N/A)	03/23/2026 19:57	**Venous pO2(T)**	39.4 mmHg (N/A)	03/23/2026 19:57
Venous HCO3	23.0 mmol/L (Normal)	03/23/2026 19:57	**Ven Base Excess**	-1.3 mmol/L (Normal)	03/23/2026 19:57
Ven sO2	75 % (High)	03/23/2026 19:57	**Ven tHb**	10.6 gm/dL (Low)	03/23/2026 19:57
Ven Hct	32 % (Low)	03/23/2026 19:57	**POC Pt Temp**	37.0 DegC (N/A)	03/23/2026 19:57
POC FIO2	21 % (N/A)	03/23/2026 19:57	**POC Sample**	Venous (N/A)	03/23/2026 19:57

Discharge Summary

POC Site	Nurse Collect (N/A)	03/23/2026 19:57	**POC Allen Test**	N/A (N/A)	03/23/2026 19:57
POC Vent Mode	NO VENT (N/A)	03/23/2026 19:57			

Cardiac

Troponin TNIH	13.34 ng/L (Normal)	03/23/2026 19:15			

General Chemistry

Glucose Level	88 mg/dL (Normal)	03/25/2026 01:20	**POC Glucose**	135 mg/dL (High)	03/24/2026 18:03
Capillary Blood Glucose POC Result	135 mg/dL (High)	03/24/2026 18:04	**Sodium**	139 mmol/L (Normal)	03/25/2026 01:20
Ven Na	138 mmol/L (Normal)	03/23/2026 19:57	**Potassium**	4.0 mmol/L (Normal)	03/25/2026 01:20
Ven K	3.4 mmol/L (Low)	03/23/2026 19:57	**Chloride**	108 mmol/L (High)	03/25/2026 01:20
CO2	19 mmol/L (Low)	03/25/2026 01:20	**Anion Gap**	12 mmol/L (Normal)	03/25/2026 01:20
BUN	12 mg/dL (Normal)	03/25/2026 01:20	**Creatinine**	0.82 mg/dL (Normal)	03/25/2026 01:20
BUN/Creat Ratio	15 (Normal)	03/25/2026 01:20	**Calcium**	8.8 mg/dL (Normal)	03/25/2026 01:20
Ven iCa	1.01 mmol/L (Low)	03/23/2026 19:57	**Albumin. Level**	3.7 gm/dL (Normal)	03/25/2026 01:20
TP	5.9 gm/dL (Normal)	03/25/2026 01:20	**A/G Ratio**	1.7 (Normal)	03/25/2026 01:20
T Bili	0.3 mg/dL (Normal)	03/25/2026 01:20	**D Bili**	0.1 mg/dL (Normal)	03/25/2026 01:20
Alk Phos	77 units/L (Normal)	03/25/2026 01:20	**AST**	15 units/L (Normal)	03/25/2026 01:20
ALT	9 units/L (Low)	03/25/2026 01:20	**Mg Lvl**	1.9 mg/dL (Normal)	03/25/2026 01:20
Phosphorus	5.3 mg/dL (High)	03/25/2026 01:20	**Estimated Creatinine Clearance**	46.46 mL/min ()	03/25/2026 03:13
eGFR Cr	72 mL/min/1.73m2 (N/A)	03/25/2026 01:20	**eGFR Pediatric**	Not Reported (N/A)	03/25/2026 01:20
Lactic Acid Lvl	1.4 mmol/L (Normal)	03/24/2026 06:38	**Hgb A1C**	4.8 % (Normal)	03/23/2026 19:15
Iron	25 mcg/dL (Low)	03/23/2026 19:15	**Total Iron Binding Capacity**	124 mcg/dL (Low)	03/23/2026 19:15
Transferrin Saturation	20 % (Normal)	03/23/2026 19:15	**Ferritin Lvl**	116.3 ng/mL (Normal)	03/23/2026 19:15
MAR Glucose Result	91 ()	03/25/2026 09:52			

General Coagulation

PT	12.8 Seconds (High)	03/23/2026 19:15	**INR**	1.2 (High)	03/23/2026 19:15
aPTT	30 Seconds (Normal)	03/23/2026 19:15	**Heparin Anti-Xa (UFH)**	<0.10 Intl_units/mL (Low)	03/23/2026 19:15

General Hematology

WBC	7.90 x10e3/mcL (Normal)	03/25/2026 01:20	**RBC**	3.46 x10e6/mcL (Low)	03/25/2026 01:20
Hgb	9.9 gm/dL (Low)	03/25/2026 01:20	**Hct**	33.4 % (Normal)	03/25/2026 01:20
MCV	96.5 Femtoliters (Normal)	03/25/2026 01:20	**MCH**	28.6 pg (Normal)	03/25/2026 01:20

Discharge Summary

MCHC	29.6 gm/dL (Low)	03/25/2026 01:20	**RDW-SD**	50.3 Femtoliters (High)	03/25/2026 01:20
RDW-CV	14.2 % (Normal)	03/25/2026 01:20	**Plt**	236 x10e3/mcL (Normal)	03/25/2026 01:20
MPV	9.4 Femtoliters (Normal)	03/25/2026 01:20	**Imm Plt Fract**	1.0 % (Low)	03/24/2026 06:40
Neut % Auto	78.9 % (N/A)	03/25/2026 01:20	**Lymph % Auto**	11.5 % (N/A)	03/25/2026 01:20
Mono % Auto	7.2 % (N/A)	03/25/2026 01:20	**Eos % Auto**	1.4 % (N/A)	03/25/2026 01:20
Baso % Auto	0.5 % (N/A)	03/25/2026 01:20	**Immature Grans %**	0.5 % (Normal)	03/25/2026 01:20
Neut # Auto	6.23 x10e3/mcL (Normal)	03/25/2026 01:20	**Lymph # Auto**	0.91 x10e3/mcL (Normal)	03/25/2026 01:20
Mono # Auto	0.57 x10e3/mcL (Normal)	03/25/2026 01:20	**Eos # Auto**	0.11 x10e3/mcL (Normal)	03/25/2026 01:20
Baso # Auto	0.04 x10e3/mcL (Normal)	03/25/2026 01:20	**Immature Grans # Auto**	0.04 x10e3/mcL (Normal)	03/25/2026 01:20

Other Chemistry

Beta Hydroxybutyrate Lvl	5.9 mg/dL (High)	03/23/2026 20:33			

Other Microbiology

MRSA by PCR	Detected (Abnormal)	03/24/2026 09:47			

Thyroid

T3 Free	2.33 pg/mL (Normal)	03/23/2026 19:15	**T4 Free**	0.94 ng/dL (Normal)	03/23/2026 19:15
TSH Ultrasensitive	2.821 mc Intl units/mL (Normal)	03/23/2026 19:15			

UA Macroscopic

Ur Color	Yellow (Normal)	03/24/2026 09:47	**Ur Clarity**	Cloudy (Abnormal)	03/24/2026 09:47
UR Glucose	Negative (Normal)	03/24/2026 09:47	**Ur Bili**	Negative (Normal)	03/24/2026 09:47
Ur Ketones	Negative (Normal)	03/24/2026 09:47	**Ur Spec Grav**	1.010 (Normal)	03/24/2026 09:47
Ur Blood	2+ (Abnormal)	03/24/2026 09:47	**Ur pH**	6.0 (Normal)	03/24/2026 09:47
Ur Protein	1+ (Abnormal)	03/24/2026 09:47	**Ur Urobilinogen**	<2.0 mg/dL (Normal)	03/24/2026 09:47
Ur Nitrite	Positive (Abnormal)	03/24/2026 09:47	**Ur Leuk Est**	2+ (Abnormal)	03/24/2026 09:47
UA Culture?	Yes (N/A)	03/24/2026 09:47	**Signs/Symptoms of UTI?**	Low Ur Symptom (N/A)	03/24/2026 09:47
UTI Other:	abdominal pain (N/A)	03/23/2026 14:58			

UA Microscopic

UA WBC Inst	29 /HPF (High)	03/24/2026 09:47	**UA RBC Inst**	37 /HPF (High)	03/24/2026 09:47
Ur Bacteria	1+ /HPF (Abnormal)	03/24/2026 09:47	**UA Squam Epi Inst**	1 /HPF (Normal)	03/24/2026 09:47
Ur Mucous	Rare /LPF (Abnormal)	03/23/2026 14:58	**Ur Hyal Cast**	1 /LPF (Normal)	03/24/2026 09:47

Discharge Summary

Urine Chemistry

Ur Sodium	34 mmol/L (N/A)	03/24/2026 09:47	**Ur Protein/Creatinine**	1.4 (High)	03/24/2026 09:47
Ur Total Protein	63.3 mg/dL (High)	03/24/2026 09:47	**Ur Creat**	44.9 mg/dL (N/A)	03/24/2026 09:47
Ur Creatinine	44.3 mg/dL (N/A)	03/24/2026 09:47	**U Microalbumin**	7.5 mg/dL (N/A)	03/24/2026 09:47
Microalb/Crea Ratio	164.9 mg/gm (N/A)	03/24/2026 09:47			

Microbiology

Culture Urine **In-Lab**

Source: U CleanCatch Body Site:
Collected Dt/Tm: 03/24/2026 09:47 Last Updated Dt/Tm: 03/24/2026 10:50

Allergies
Allergies (Active and Proposed Allergies Only)
Adhesive Bandage (Severity: Moderate, Onset: Unknown)
Reactions: Itching of skin

Objective

Vital Signs (24 hrs)	**Last Charted**		**Minimum**		**Maximum**	
Temp	36.5	03/25/2026 07:46	36.8	03/24/2026 19:58	37.1	03/24/2026 23:10
Apical Heart Rate	83	03/25/2026 09:36	72	03/24/2026 20:28	83	03/25/2026 09:36
Resp Rate	18	03/25/2026 07:46	17	03/25/2026 04:37	20	03/24/2026 12:38
SBP	133	03/25/2026 07:46	124	03/24/2026 18:00	**H** 173	03/24/2026 11:00
DBP	64	03/25/2026 07:46	64	03/25/2026 07:46	**H** 82	03/24/2026 14:00
MAP	91	03/25/2026 07:46	91	03/24/2026 18:00	112	03/24/2026 11:00
SpO2	94	03/25/2026 07:46	94	03/25/2026 07:46	100	03/24/2026 11:00
O2 Therapy	Room air		Room air		Room air	

Intake and Output (current encounter)

	03/23 12:09 03/25 10:31	**03/25 07:00 03/25 10:31**	**03/24 07:00 03/25 06:59**	**03/23 07:00 03/24 06:59**	**03/22 07:00 03/23 06:59**
Intake	700.0	0	500.0	200	0
Output	0	0	0	0	0
Net Total	700.0	0	500.0	200	0
Urine Count	4	2	2	0	0

Discharge Summary

Physical Exam:
General: 81yo Female laying in bed in no acute distress
Mental Status: Alert and oriented to person, place and time. Normal affect.
HEENT: Normocephalic, Atraumatic. PERRL. EOMI. Normal dentition. Oropharynx clear, mucous membranes moist. Ears and nose without lesions or deformities. Full range of motion. Trachea midline.
Respiratory: CTA bilaterally, no crackles, rales, wheezes, rhonchi.
Cardiovascular: Regular rate and rhythm, no murmurs, rubs or gallops, normal S1, S2, no peripheral edema.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Bowel sounds normal.
Genitourinary: No CVA tenderness, No suprapubic tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Hospital Course
81-year-old female with past medical history of abnormal uterine bleeding and a fib on anticoagulation, was scheduled for hysterectomy was found to have a low potassium at her PCP prior to scheduled surgery was brought to Valley Hospital Medical Center on 3/23/2026 to treat hypokalemia. Patient surgery was rescheduled for the following week. Upon admission patient hemodynamically stable, saturating well on room air. Potassium elevated 2.4 EKG showed atrial fibrillation. Patient is not on anticoagulation for her A-fib due to severe GI bleeding previously. Patient required multiple doses of IV and oral potassium to replete levels reading of 4.0. PT OT evaluated the patient and determined that patient would benefit from acute rehab following discharge to improve functional deficits. Recommended the patient follow-up with her PCP regarding hypokalemia.

On discharge, patient has stable vitals, labs within normal limits, and acceptable physical exam. Patient denies any symptoms at this time. Patient was advised to follow-up with PCP within the next 1-2 weeks. Patient is to return to ED if symptoms worsen or if there are any questions or concerns. All questions and concerns were answered and addressed at bedside. Patient is currently alert and oriented to person, place, time, situation. Risk, benefits and alternatives conveyed. Patient has verbalized understanding of diagnoses, management, treatment, and follow-up instructions. Patient is safe for discharge at this time.

Greater than 30 minutes were spent in the examination, evaluation, counseling, and review of medications and discharge plan for this patient.

This patient will be discussed with the attending physician. Further recommendations to follow.

Trevor Orr, MD
PGY-3 Family Medicine
Valley Hospital Medical Center

Electronically Signed By: Orr, Trevor MD
On: 03.25.2026 10:32 PDT

Electronically Signed On: 03.25.2026 11:42 PDT
Techathaveewat, Worawith DO